DREAMS THAT COME FROM SCIENCE

Menopur®
menotrophin

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FERRING
PHARMACEUTICALS

OUTCOMES

- EMBRYO QUALITY.[1,2]
- ENDOCRINE PROFILE.[3]
- LIVE BIRTH RATES.[1,4–6,§]

BALANCING SCIENCE AND DREAMS

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OUTCOMES

EVIDENCE OF EXCELLENCE[1, 4-6]

LIVE BIRTH RATES	ONGOING PREGNANCY RATES

EISG	MERIT	MEGASET	MEGASET-HR ***
MENOPUR: **21.4%**	MENOPUR: **26%**	MENOPUR: **29%**	MENOPUR: **52.2%**
rFSH alfa:* 18.9%	rFSH alfa:* 22%	rFSH beta:+ 26%	rFSH alfa:* 48.7%

Live birth rate was either a secondary or post-trial outcome from non-inferior phase 3/4 RCTs. ***See study references***

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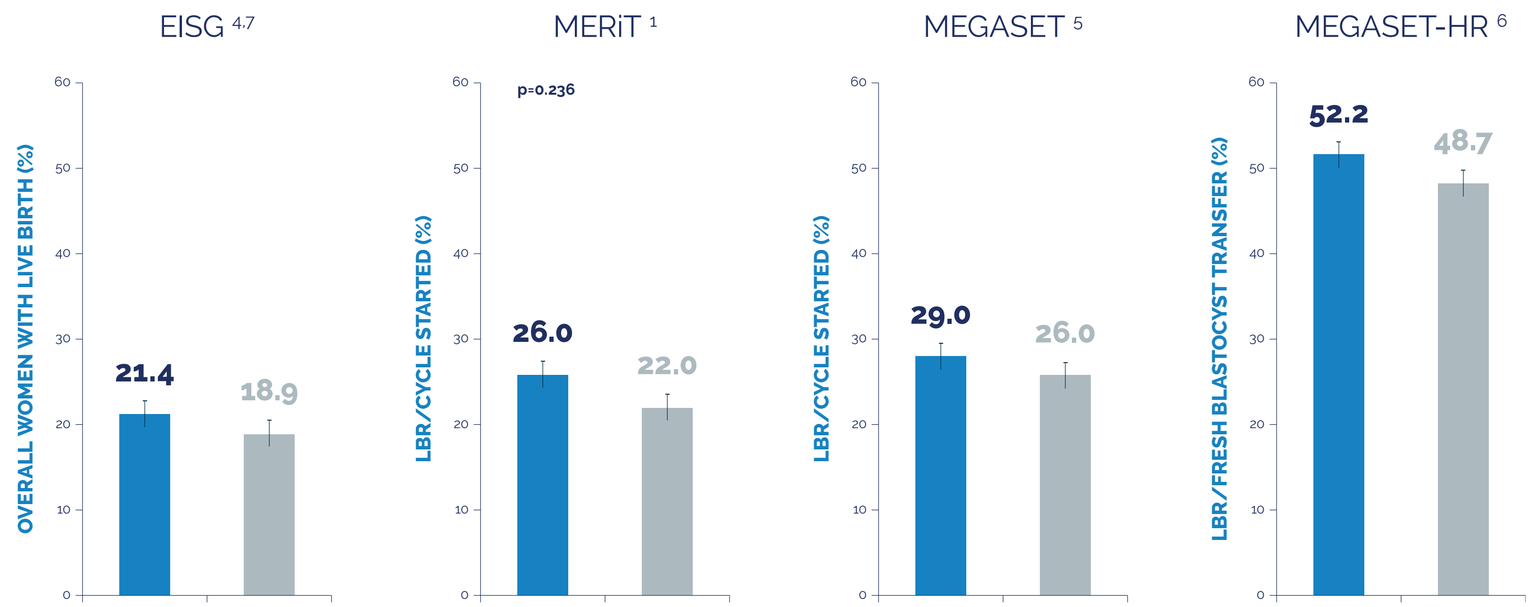
CONSISTENT LIVE BIRTH RATES ACROSS FOUR LARGE RANDOMISED CLINICAL TRIALS[1, 4-7]



POPULATION	APT: 373	354	ITT: 363	368	PP: 343	333	mITT: 310	309
STARTING DOSE	225 IU	225 IU	225 IU	225 IU	150 IU	150 IU	150 IU	150 IU
	LONG AGONIST PROTOCOL				GnRH ANTAGONIST PROTOCOL			

■ **MENOPUR** ■ **rFSH** alfa* or beta†

Live birth rate was either a secondary or post-trial outcome from non-inferior phase 3/4 RCTs.
See study references

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OUTCOMES

EVIDENCE OF EXCELLENCE[1, 5-7]

LIVE BIRTH RATES	ONGOING PREGNANCY RATES		
EISG	MERIT	MEGASET	MEGASET-HR
MENOPUR: **25%**	MENOPUR: **27%**	MENOPUR: **30%**	MENOPUR: **35.5%**
rFSH alfa:* 22%	rFSH alfa:* 22%	rFSH beta:+ 27%	rFSH alfa:* 30.7%

Non-inferior phase 3/4 RCTs. ***See study references***

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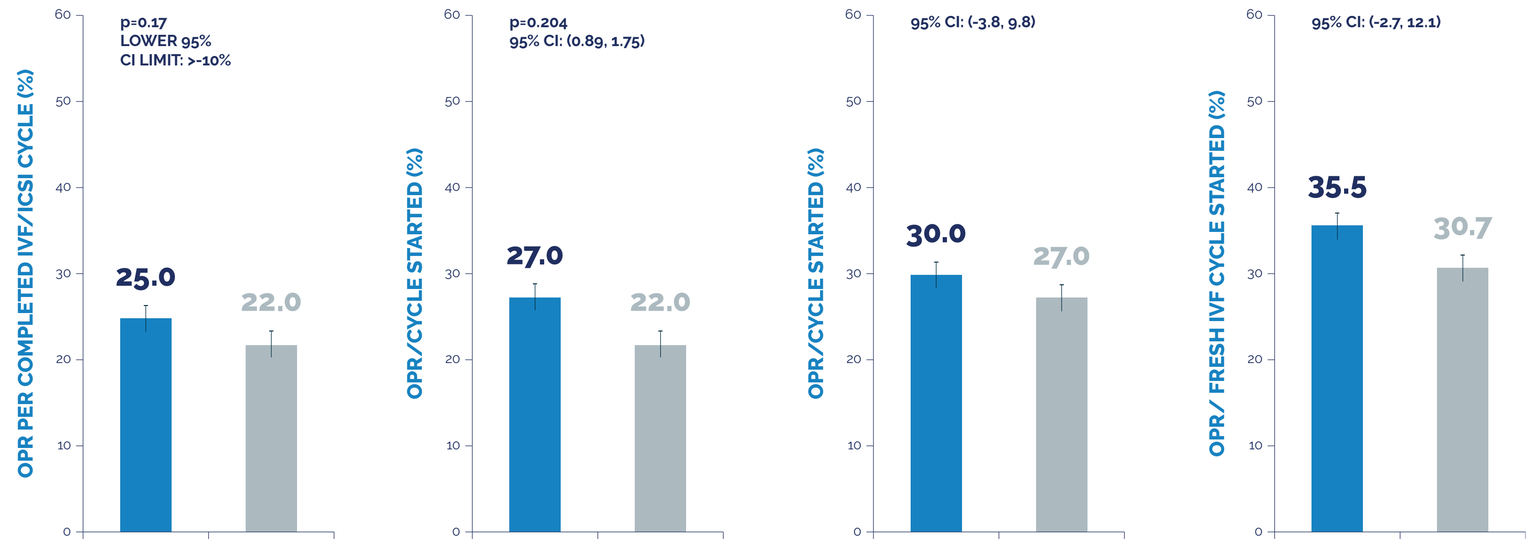
CONSISTENT ONGOING PREGNANCY RATES ACROSS FOUR LARGE RANDOMISED CLINICAL TRIALS[1, 5-7]



	LONG AGONIST PROTOCOL				GnRH ANTAGONIST PROTOCOL			
POPULATION	APT: 344	317	ITT: 363	368	APT: 343	333	mITT: 310	309
STARTING DOSE	225 IU	225 IU	225 IU	225 IU	150 IU	150 IU	150 IU	150 IU

■ **MENOPUR** ■ **rFSH** alfa* or beta†

Non-inferior phase 3/4 RCTs. Graphs adapted from published studies. ***See study references***

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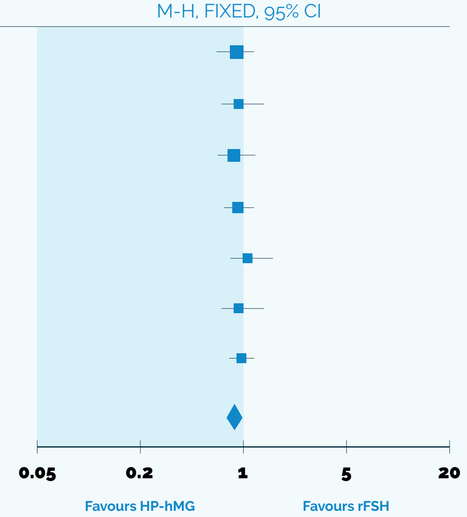
OUTCOMES

RESULTS UNDER INDEPENDENT ANALYSIS[8]

Results from an independent meta-analysis of randomised controlled trials that compared **MENOPUR** with rFSH alfa* and beta[8†]

LIVE BIRTH RATES | **CLINICAL PREGNANCY RATES**

STUDY OR SUB-GROUP	rFSH EVENTS	rFSH TOTAL	HP-hMG EVENTS	HP-hMG TOTAL	WEIGHT	M-H, FIXED, 95% CI	rFSH
EISG 2002	78	386	98	395	18.8 %	0.81 [0.63, 1.06]	**Alfa**
KILANI 2003	28	50	30	50	5.8 %	0.93 [0.67, 1.30]	**Alfa**
ANDERSEN 2006	87	368	100	363	19.5 %	0.86 [0.67, 1.10]	**Alfa**
BOSCH 2008	55	140	59	140	11.4 %	0.93 [0.70, 1.24]	**Alfa**
HOMPES 2008	77	317	76	312	14.9 %	1.00 [0.76, 1.31]	**Alfa/ Beta**
YE 2012	33	64	36	63	7.0 %	0.90 [0.66, 1.24]	**Alfa**
DEVROEY 2012	106	375	116	374	22.5 %	0.91 [0.73, 1.14]	**Beta**
TOTAL (95% CI)		**1700**		**1697**	**100.0%**	**0.90 [0.81, 1.00]**	



Heterogeneity: $Chi^2 = 1.35$, df=6 (p=0.097); $I^2 = 0\%$
Test for overall effect: Z=2.02 (p=0.04)

The meta-analysis aimed to assess the total amount of HP-hMG or rFSH used per woman that started an IVF/ICSI cycle.
See study references

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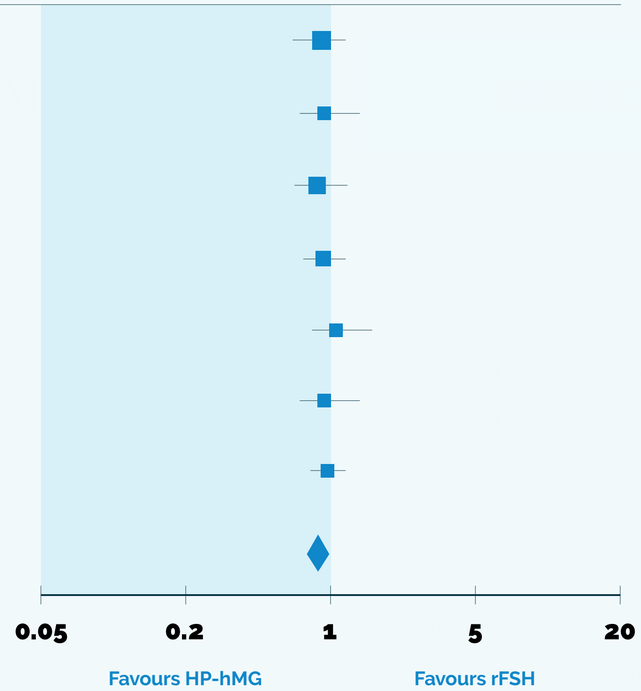
CONSISTENT CLINICAL PREGNANCY RATES WHEN COMPARED WITH rFSH ALFA* AND BETA' ‡

STUDY OR SUB-GROUP	rFSH
EISG 2002	Alfa
KILANI 2003	Alfa
ANDERSEN 2006	Alfa
BOSCH 2008	Alfa
HOMPES 2008	Alfa/ Beta
YE 2012	Alfa
DEVROEY 2012	Beta
TOTAL (95% CI)	



Heterogeneity: Chi² = 1.35, df=6 (p=0.097); I² = 0%
Test for overall effect: Z=2.02 (p=0.04)

The meta-analysis aimed to assess the total amount of HP-hMG or rFSH used per woman that started an IVF/ICSI cycle.
See study references

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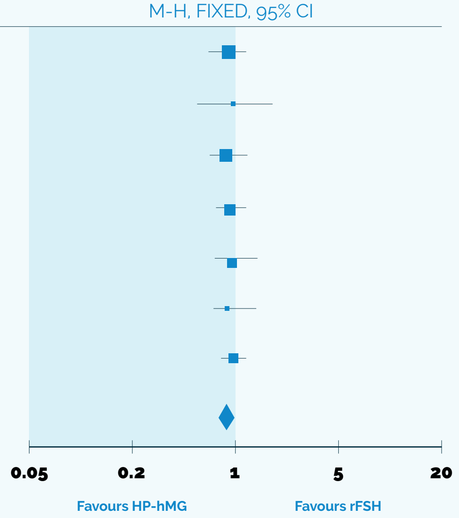
OUTCOMES

RESULTS UNDER INDEPENDENT ANALYSIS[8]

Significantly higher live birth rates were shown in an independent meta-analysis of randomised controlled trials that compared rFSH alfa* and beta† with **MENOPUR**[8]

LIVE BIRTH RATES | CLINICAL PREGNANCY RATES

	rFSH		HP-hMG				
STUDY OR SUB-GROUP	EVENTS	TOTAL	EVENTS	TOTAL	WEIGHT	M-H, FIXED, 95% CI	rFSH
EISG 2002	78	386	80	395	17.7 %	0.86 [0.64, 1.15]	**Alfa**
KILANI 2003	28	50	12	50	2.7 %	0.92 (0.45, 1.88]	**Alfa**
ANDERSEN 2006	87	368	97	363	21.9 %	0.83 (0.65, 1.08]	**Alfa**
BOSCH 2008	55	317	74	312	16.7 %	0.94 [0.71, 1.26]	**Alfa**
HOMPES 2008	77	140	48	140	10.7 %	0.92 [0.66, 1.28]	**Alfa/ Beta**
YE 2012	33	64	28	63	6.3 %	0.67 [0.42, 1.07]	**Alfa**
DEVROEY 2012	106	375	107	374	24.0 %	0.93 [0.74, 1.18]	**Beta**
TOTAL (95% CI)		**1700**		**1697**	**100.0%**	**0.88 [0.78, 0.99]**	



Heterogeneity: $Chi^2 = 1.35$, df=6 (p=0.097); $I^2 = 0\%$
Test for overall effect: Z=2.02 (p=0.04)

The meta-analysis aimed to assess the total amount of HP-hMG or rFSH used per woman that started an IVF/ICSI cycle.
See study references

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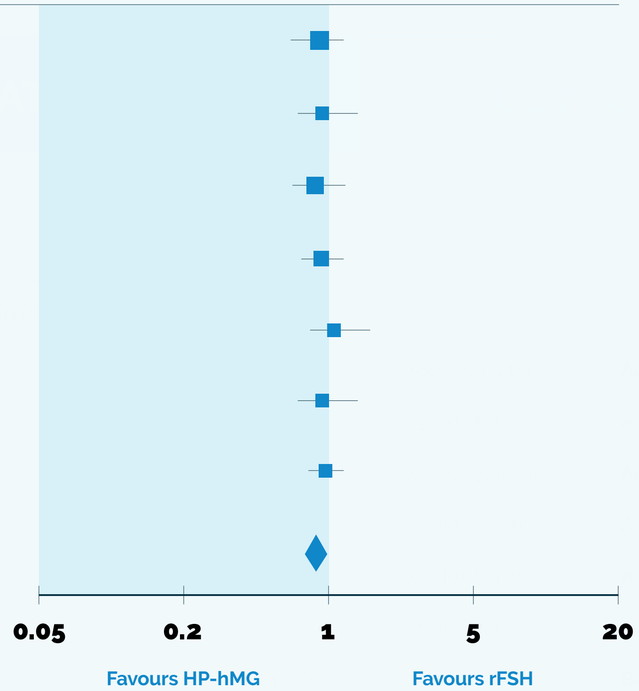
CONSISTENT CLINICAL PREGNANCY RATES WHEN COMPARED WITH rFSH ALFA* AND BETA' ǂ

STUDY OR SUB-GROUP	rFSH	RISK RATIO M-H, FIXED, 95% CI
EISG 2002	Alfa	
KILANI 2003	Alfa	
ANDERSEN 2006	Alfa	
BOSCH 2008	Alfa	
HOMPES 2008	Alfa/ Beta	
YE 2012	Alfa	
DEVROEY 2012	Beta	
TOTAL (95% CI)		



Heterogeneity: Chi² = 1.35, df=6 (p=0.097); I² = 0%
Test for overall effect: Z=2.02 (p=0.04)

The meta-analysis aimed to assess the total amount of HP-hMG or rFSH used per woman that started an IVF/ICSI cycle.
See study references

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OUTCOMES

LIVE BIRTH RATES IN FROZEN TRANSFERS

In the MEGASET-HR trial, MENOPUR's favourable live birth rate was observed in frozen as well as fresh transfers [6,9]

FET1 (FROZEN)	FET2 (FROZEN)*	ANY FET (FROZEN)*
MENOPUR: 57.3%	**MENOPUR: 62.5%**	**MENOPUR: 63.4%**
rFSH alfa:* 45.4%	rFSH alfa:* 50.0%	rFSH alfa:* 50.8%

Live birth rate was post-trial outcome from a non-inferior phase 4 RCT. ***See study references***

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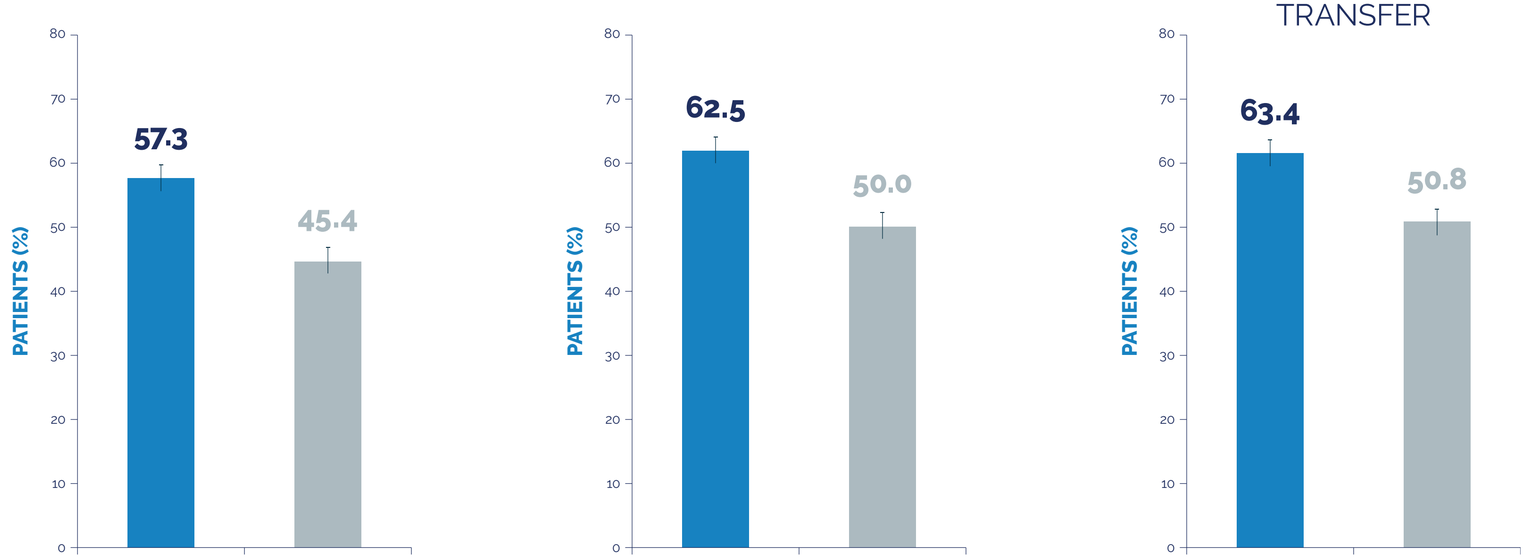
LIVE BIRTH RATES IN FROZEN TRANSERS IN PREDICTED HIGH RESPONDERS [6,9]



	LBR: FET1		LBR: FET2		LBR: ANY FROZEN TRANSFER	
POPULATION mITT	82	130	8	12	82	130
STARTING DOSE	150 IU	150 IU	150 IU	150 IU	150 IU	150 IU
	GnRH ANTAGONIST PROTOCOL		GnRH ANTAGONIST PROTOCOL		GnRH ANTAGONIST PROTOCOL	

■ **MENOPUR** ■ **rFSH** alfa*

Live birth rate was a post-trial outcome from a non-inferior phase 4 RCT. Graphs adapted from published studies.
See study references

Live birth rate was post-trial outcome from a non-inferior phase 4 RCT. ***See study references***

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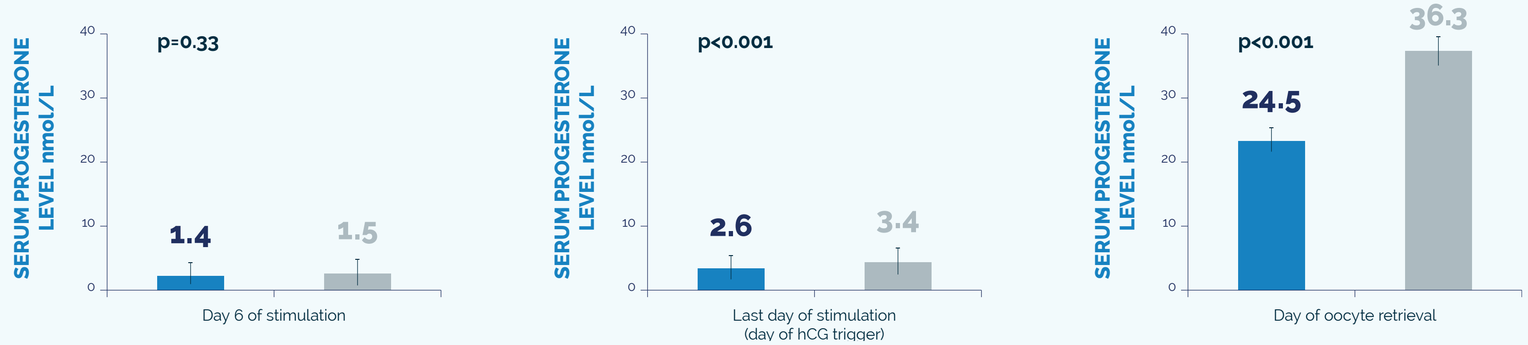
OUTCOMES

MENOPUR'S UNIQUE COMBINATION OF FSH AND HCG...

...may influence the entire ovarian endocrine

ENDOCRINE ENVIRONMENT | OOCYTE SELECTION

Lower Progesterone levels achieved with MENOPUR create a favourable stimulation environment for fresh transfer[2]



POPULATION mITT	363	368	363	368	363	368
STARTING DOSE	225 IU	225 IU	225 IU	225 IU	225 IU	225 IU
	LONG AGONIST PROTOCOL		LONG AGONIST PROTOCOL		LONG AGONIST PROTOCOL	

■ **MENOPUR** ■ **rFSH** alfa*

Endocrine profile was a secondary outcome from a non-inferiority phase 3 RCT. Graphs adapted from published study.
See study references

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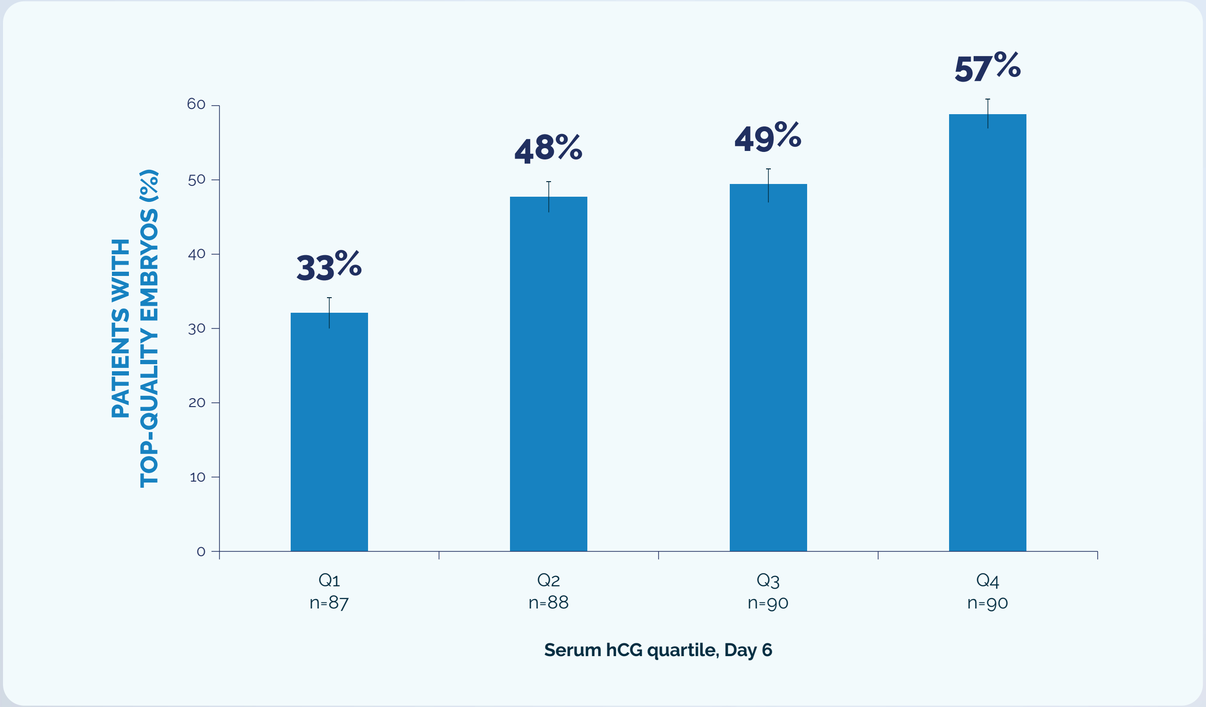
OUTCOMES

MENOPUR'S UNIQUE COMBINATION OF FSH AND HCG...

...may influence the entire ovarian endocrine environment to support oocyte selection[2]

ENDOCRINE ENVIRONMENT | **OOCYTE SELECTION**

INCREASING hCG LEVELS ARE ASSOCIATED WITH MORE TOP-QUALITY* EMBRYOS[2]



Embryo quality was a post-trial outcome from a non-inferiority phase 3 study. Graph adapted from published studies.
See study references

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OUTCOMES

DEVELOPMENT OF TOP-QUALITY EMBRYOS [1¶]

A higher proportion of oocytes developed into top-quality embryos with **MENOPUR** versus rFSH alfa* in the MERiT trial[1]

PROPORTION OF TOP QUALITY EMBRYOS PER OOCYTES RETRIEVED (%)*

MENOPUR: 11.3%

rFSH alfa:* 9.0%

Embryo quality was a secondary outcome from a non-inferior phase 3 RCT.
See study references

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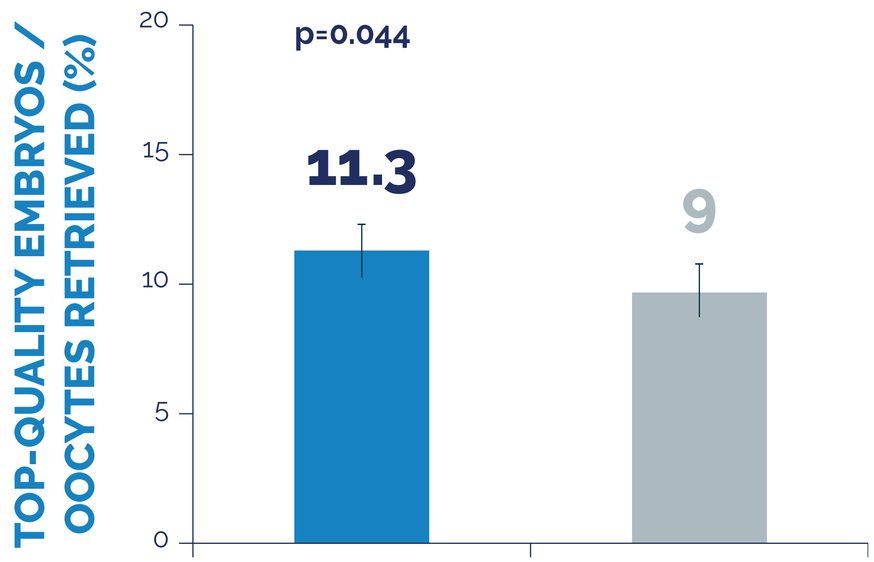
A HIGHER PROPORTION OF OOCYTES DEVELOPED INTO TOP-QUALITY EMBRYOS WITH MENOPUR VERSUS rFSH ALFA* IN THE MERiT TRIAL[1]



	MENOPUR	rFSH alfa*
POPULATION ITT	363	368
STARTING DOSE	225 IU	225 IU

LONG AGONIST PROTOCOL

Embryo quality was a secondary outcome from a non-inferior phase 3 RCT. Graph adapted from published studies.
See study references

■ **MENOPUR** ■ **rFSH** alfa*

Embryo quality was a secondary outcome from a non-inferior phase 3 RCT.
See study references

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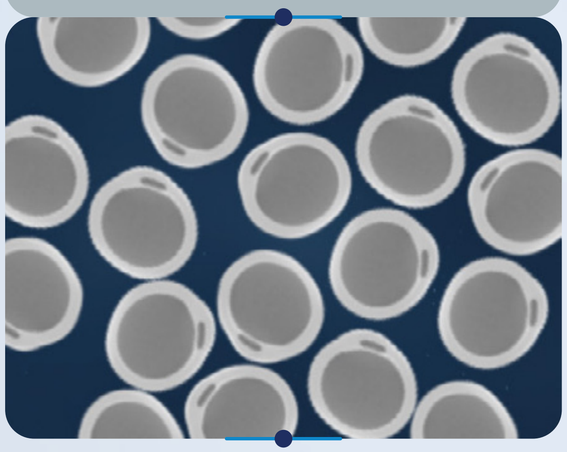
OUTCOMES

DOES NUMBER OF OOCYTES RETRIEVED REFLECT OUTCOMES?

The ovarian response in patients predicted to be high responders [6]

OOCYTE SELECTION	ENDOCRINE ENVIRONMENT

MENOPUR	rF.SH alfa*
15.1	22.2



Number of oocytes retrieved was a secondary outcome from a non-inferior phase 4 RCT.
See study references

OUTCOMES

DOES NUMBER OF OOCYTES RETRIEVED REFLECT OUTCOMES?

The ovarian response in patients predicted to be high responders does not translate into clinical outcomes that matter [6]

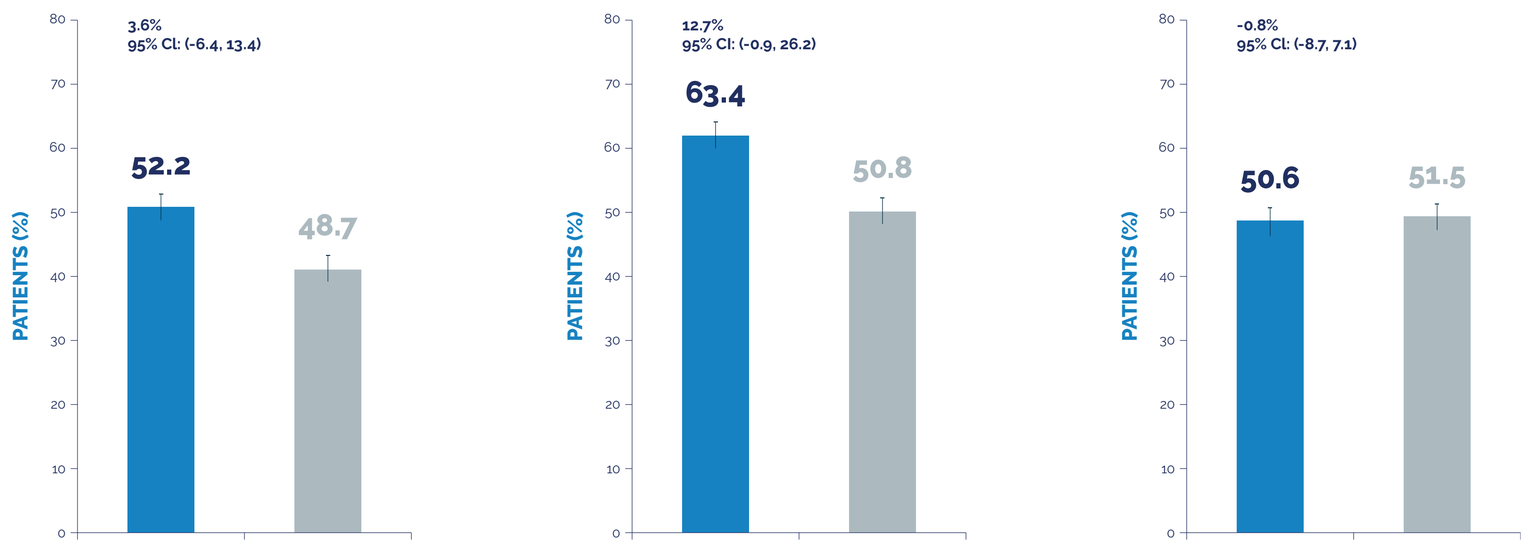
OOCYTES RETRIEVED	LIVE BIRTH OUTCOMES

LIVE BIRTH RATE (FRESH)	LIVE BIRTH RATE (FROZEN)	LIVE BIRTH RATE (FRESH + FROZEN)
MENOPUR: 52.2%	**MENOPUR: 63.4%**	**MENOPUR: 50.6%**
rFSH alfa:* 48.7%	rFSH alfa:* 50.8%	rFSH alfa:* 51.5%

43 less transfer cycles and 65 less transferred embryos were required with **MENOPUR** to achieve similar cumulative live birth rates[1]

Live birth rate was a post-trial outcome from a non-inferiority phase 4 RCT.
Also see early pregnancy loss rates in predicted high responders. See study references

LIVE BIRTH OUTCOMES IN PREDICTED HIGH RESPONDERS [6]



	LBR/FRESH BLASTOCYST TRANSFER		LBR: ANY FROZEN TRANSFER		CUMULATIVE LBR	
mITT	201	191	82	130	310	309
STARTING DOSE	150 IU	150 IU	150 IU	150 IU	150 IU	150 IU
	GnRH ANTAGONIST PROTOCOL		GnRH ANTAGONIST PROTOCOL		GnRH ANTAGONIST PROTOCOL	

■ **MENOPUR** ■ **rFSH** alfa*

Live birth rate was a post-trial outcome from a non-inferior phase 4 RCT. Graphs adapted from published studies.
See study references

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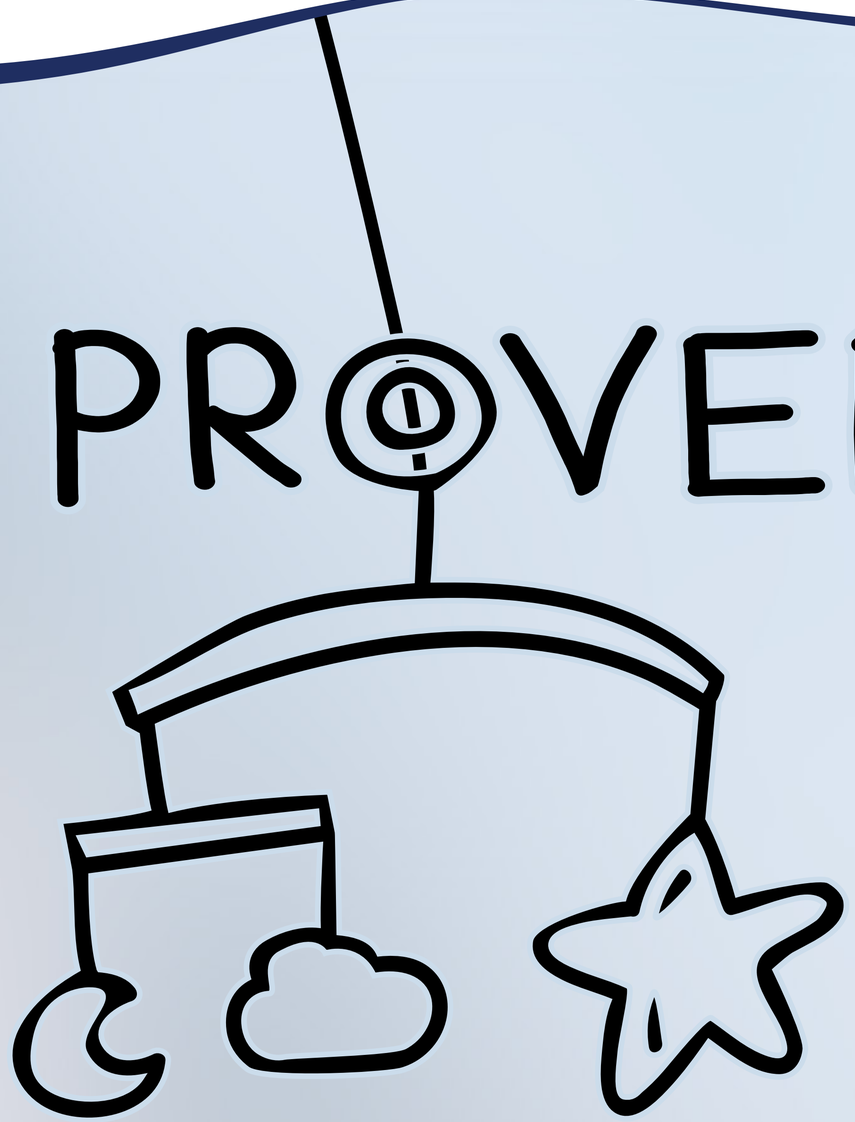
PROVEN



- FOUR LARGE RCTS.[1, 5–7]
- SUPPORTED BY SYSTEMATIC META-ANALYSIS.[8]
- APPROVED IN OVER 130 COUNTRIES.[10]

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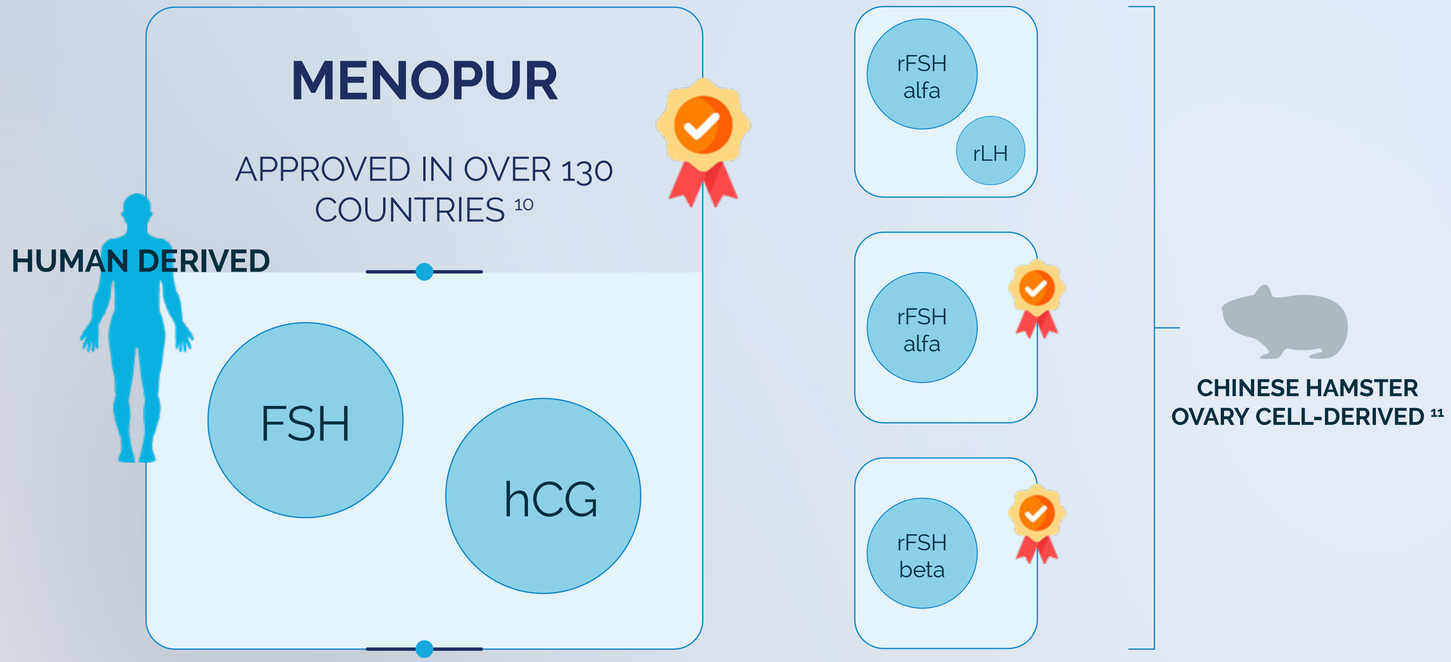
PROVEN

NOT ALL GONADOTROPHINS ARE THE SAME

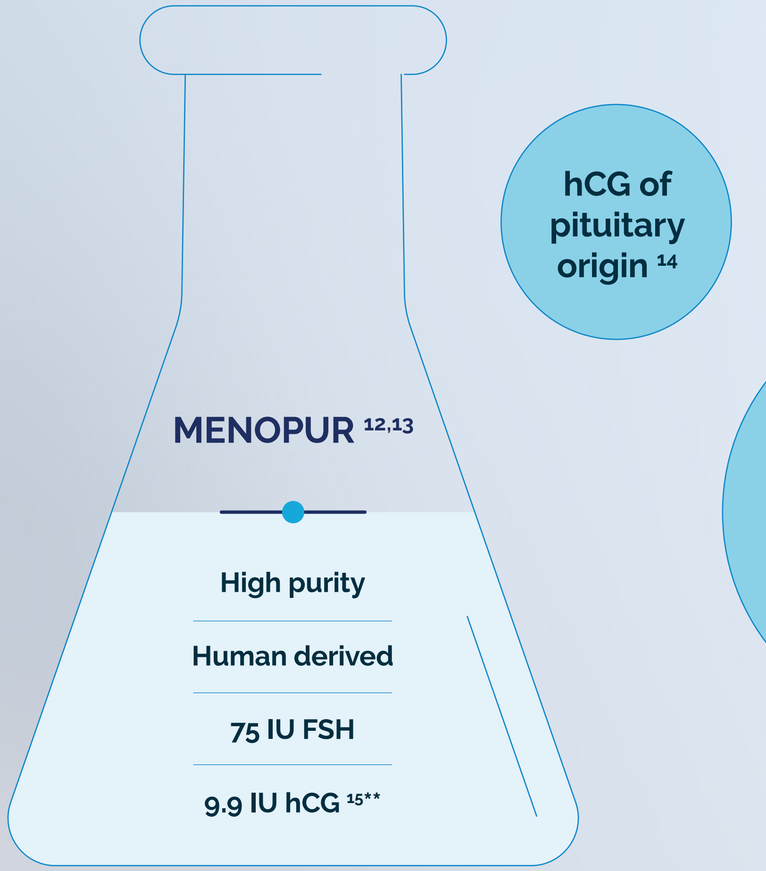
MENOPUR: driving quality with highly purified human FSH and hCG



PROVEN

NOT ALL MENOTROPHINS ARE THE SAME

MENOPUR has a unique and known composition



hCG of pituitary origin [14]

Pituitary hCG mimics LH action in driving follicular maturation at a 40-fold greater potency [14]

PROVEN

EXPANDING EVIDENCE ACROSS 4 LARGE RCTS

2002 PHASE 3	2006 PHASE 3	2012 PHASE 3	2020 PHASE 4
EISG [7]	MERIT [1]	MEGASET [5]	MEGASET-HR [6]
MENOPUR vs rFSH alfa*	**MENOPUR** vs rFSH alfa*	**MENOPUR** vs rFSH beta†	**MENOPUR** vs rFSH alfa*
Total participants **781**	Total participants **731**	Total participants **749**	Total participants **619**

TOTAL NUMBER OF PATIENTS ENROLLED IN LARGE PHASE 3 AND PHASE 4 **MENOPUR** RCTS: **2,880**

The primary outcome in all listed non-inferior phase 3/4 RCTs was ongoing pregnancy rate. ***See study references***

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CONFIDENCE

- CONSISTENT PREGNANCY OUTCOMES.[1, 5–7#]
- FAVOURABLE SAFETY PROFILE.[1, 5–7 ||]
- TWO DECADES' EXPERIENCE.[10]

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DREAMS THAT COME
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CONFIDENCE

FAVOURABLE SAFETY PROFILE

Comparable rates of OHSS § were observed with **MENOPUR** and rFSH alfa* and betat in general population [1, 5,7]

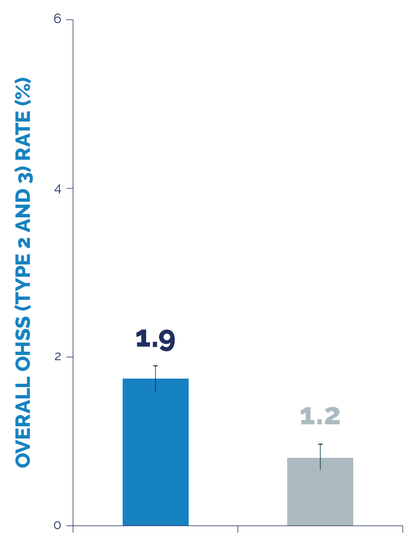
Significantly reduced rates of OHSS were observed with MENOPUR compared to rFSH alfa* in predicted high responders [6]

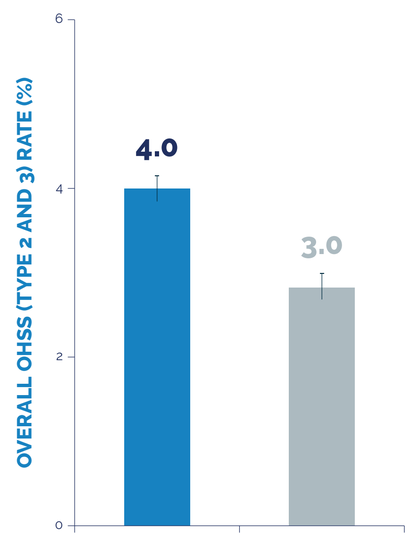
EISG	MERIT	MEGASET	MEGASET-HR
MENOPUR: **1.9%**	MENOPUR: **4.0%**	MENOPUR: **3.0%**	MENOPUR: **9.7%**
rFSH alfa:* 1.2%	rFSH alfa:* 3.0%	rFSH beta:+ 3.0%	rFSH alfa:* 21.4%
p=NS	p=NS	p=NS	95% CI: (-17.3, -6.1)

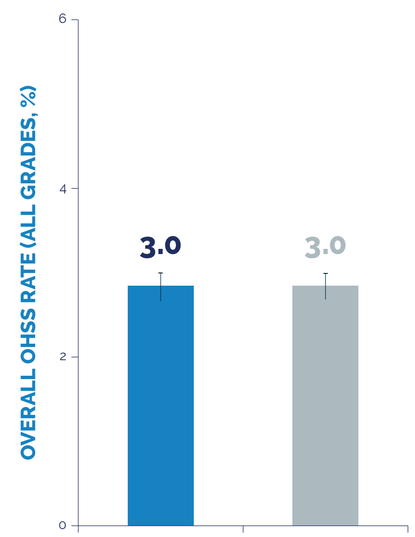
Safety was a secondary outcome from non-inferior phase 3/4 RCTs. ***See study references***

OHSS RATES IN GENERAL POPULATION [1,5,7]

Comparable rates of OHSS were observed with MENOPUR and rFSH [1,5,7]







POPULATION	APT:	373	354	SAFETY:	363	368	SAFETY:	374	375
STARTING DOSE		225 IU	225 IU		225 IU	225 IU		150 IU	150 IU
	LONG AGONIST PROTOCOL			LONG AGONIST PROTOCOL			GnRH ANTAGONIST PROTOCOL		

■ **MENOPUR** ■ **rFSH** alfa* or beta†

Safety was a secondary outcome from a non-inferior phase 4 RCT. Graphs adapted from published studies.
See study references

Safety was a secondary outcome from non-inferior phase 3/4 RCTs. ***See study references***

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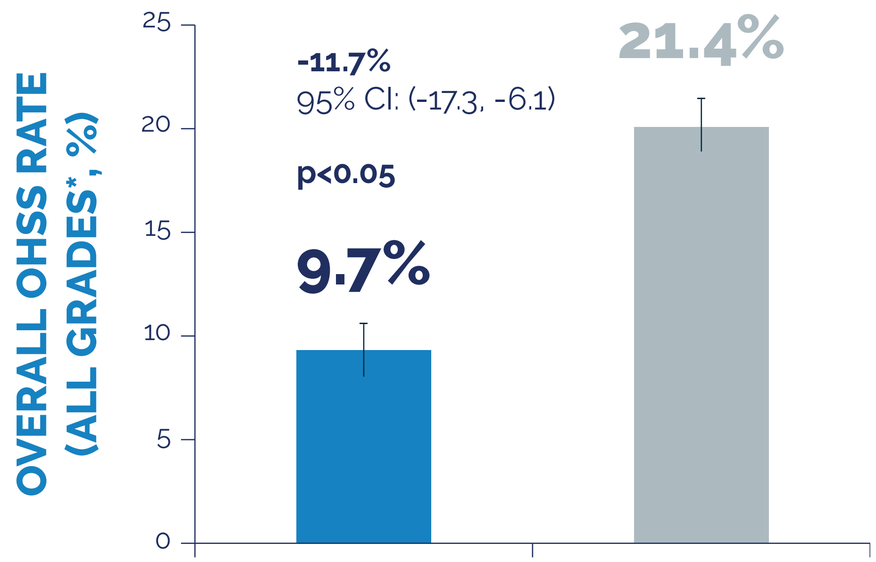
OHSS RATES IN PREDICTED HIGH RESPONDERS [6]

OHSS incidence compared with rFSH alfa* in the MEGASET-HR trial [6,†]



SAFETY POPULATION	310	309
STARTING DOSE	150 IU	150 IU
GRH ANTAGONIST PROTOCOL		

Safety was a secondary outcome from a non-inferior phase 4 RCT. Graph adapted from published studies.
See study references

■ **MENOPUR** ■ **rFSH** alfa*

Safety was a secondary outcome from non-inferior phase 3/4 RCTs. ***See study references***

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DREAMS THAT COME FROM SCIENCE

CONFIDENCE

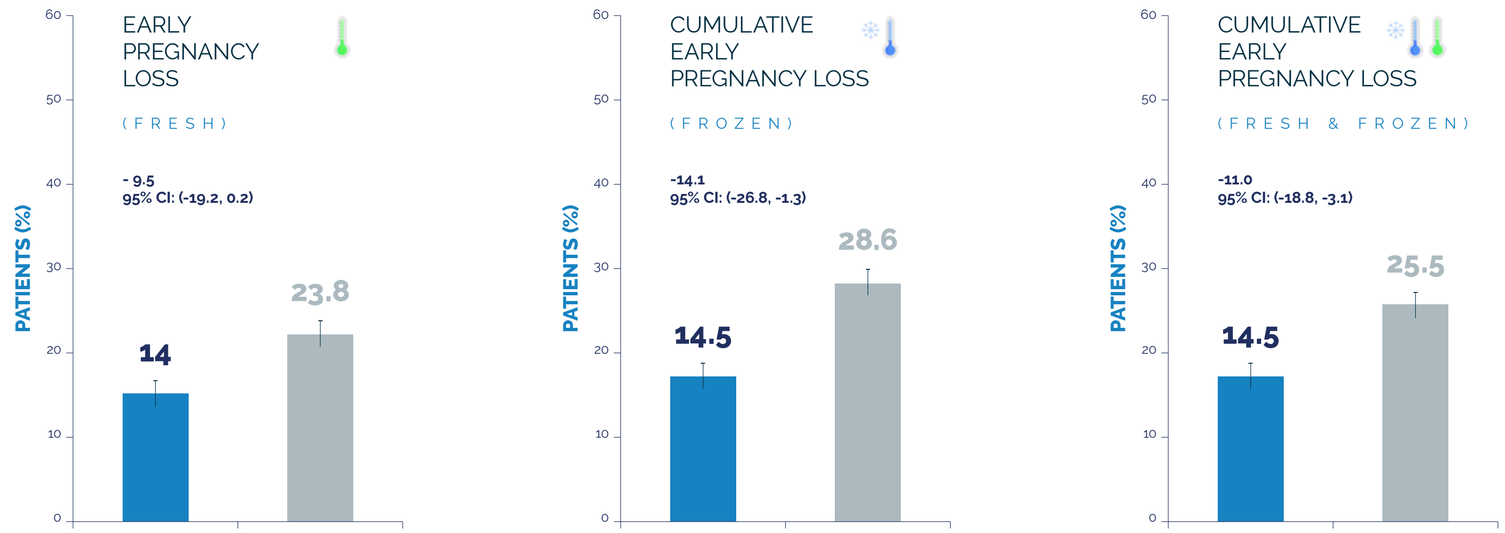
EARLY PREGNANCY LOSS RATES IN PREDICTED HIGH RESPONDERS [6]

Cumulatively, pregnancy loss in fresh and frozen cycles was lower following **MENOPUR** treatment compared with rFSH alfa* in the MEGASET-HR trial [6]

EARLY PREGNANCY LOSS (FRESH)	CUMULATIVE EARLY PREGNANCY LOSS (FROZEN)	CUMULATIVE EARLY PREGNANCY LOSS (FRESH & FROZEN)
MENOPUR: 14.0%	**MENOPUR: 14.5%**	**MENOPUR: 14.5%**
rFSH alfa:* 23.8%	rFSH alfa:* 28.6%	rFSH alfa:* 25.5%

Early pregnancy loss was a post-trial outcome from a non-inferior phase 4 RCT. ***See study references***

MENOPUR EARLY PREGNANCY LOSS RATES IN FRESH AND ROZEN TRANSFERS IN PREDICTED HIGH RESPONDERS [6]



POPULATION mITT	126	122	62	91	186	204
STARTING DOSE	150 IU	150 IU	150 IU	150 IU	150 IU	150 IU
	GRH ANTAGONIST PROTOCOL		GnRH ANTAGONIST PROTOCOL		GnRH ANTAGONIST PROTOCOL	

■ **MENOPUR** ■ **rFSH** alfa*

Pregnancy loss was a post-trial outcome from a non-inferior phase 4 RCT. Graphs adapted from published studies. ***See study references***

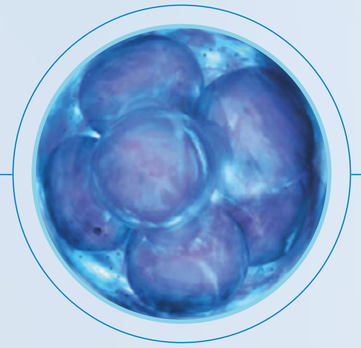
Early pregnancy loss was a post-trial outcome from a non-inferior phase 4 RCT. ***See study references***

SUMMARY.

PI

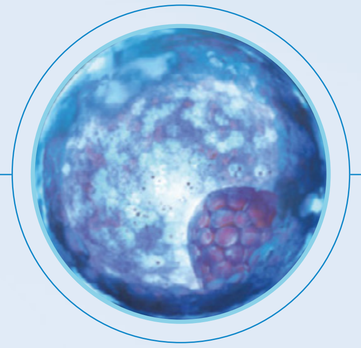
OUTCOMES

- EMBRYO QUALITY.[1,2]
- ENDOCRINE PROFILE.[3]
- LIVE BIRTH RATES. [1,4-6#]



PROVEN

- FOUR LARGE RCTS.[1, 5-7]
- SUPPORTED BY SYSTEMATIC META-ANALYSIS.[8]
- APPROVED IN OVER 130 COUNTRIES.[10]



CONFIDENCE

- CONSISTENT PREGNANCY OUTCOMES.[1, 4-7]
- FAVOURABLE SAFETY PROFILE.[1, 5-7]
- TWO DECADES' EXPERIENCE.[10]

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STUDY DESIGNS.

PI

EISG

MEGASET

MERITT

Bordewijk Meta-Analysis

MERiT Retrospective Analysis

MEGASET-HR

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DREAMS THAT COME
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STUDY DESIGNS

EISG

The European and Israeli Study Group trial was a multinational, open-label, randomised, parallel group, comparative phase 3 trial

PATIENT POPULATION	781 patients, 18-38 years of age with regular ovulatory menstrual cycles and normal uteri eligible for IVF/ICSI
RANDOMISATION	MENOPUR 225-450 IU/day (n=373) rFSH alfa (Gonal-F) 225-450 IU/day (n=354)
PRIMARY OUTCOME	**Ongoing pregnancy rate** (defined as clinical pregnancy confirmed by ultrasound at 10 weeks after ovum pickup following one IVF/ICSI cycle)
SELECT SECONDARY OUTCOMES	Number of follicles, number of oocytes retrieved and fertilised, number of embryos transferred, days of stimulation with gonadotrophin, and oestradiol levels on the first and sixth day of gonadotrophin administration and at the time of hCG administration
STIMULATION PROTOCOL	**Long agonist:** After downregulation with a GnRH agonist, all patients received a fixed dose of 225 IU/day of MENOPUR or rFSH; at Day 6, the dosage could be adjusted up to 450 IU/day depending on ovarian response
OVULATION TRIGGER	hCG (5000-10,000 IU
TRANSFER	Fresh only. One to three normally developed embryos were transferred at visit 7

STUDY DESIGNS

MERIT

The MERiT study was a randomised, assessor-blind, multinational, open-label, parallel group multicentre trial

PATIENT POPULATION	731 patients, 21-37 years of age with regular ovulatory menstrual cycles and presumed to be ovulatory
RANDOMISATION	MENOPUR 225-450 IU/day (n=363) rFSH alfa (Gonal-F) 225-450 IU/day (n=368)
PRIMARY OUTCOME	**Ongoing pregnancy rate per started cycle** (defined as the presence of at least one viable foetus 10 to 11 weeks after embryo transfer, documented by transvaginal ultrasound)
SELECT SECONDARY OUTCOMES	Clinical pregnancy rate, endometrial status, follicular development, oocytes retrieved, fertilisation rate, embryo quality, endocrine profile in serum and follicular fluid and treatment-emergent adverse events (onset after start of stimulation), moderate/severe OHSS and early pregnancy loss
STIMULATION PROTOCOL	**Long agonist:** Patients were treated with 225 IU of MENOPUR or rFSH for 5 days, following which doses were adjusted individually by <75 IU and up to 450 IU per day according to the follicular response
OVULATION TRIGGER	hCG (250 mcg)
TRANSFER	Fresh only. One or two embryos were transferred on day 3 after oocyte retrieval

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STUDY DESIGNS

MERIT RETROSPECTIVE ANALYSIS

Smitz et al., 2007:
Retrospective endocrinology analysis of the MERiT study

PATIENT POPULATION	All patients that were included in the MERiT study. See MERiT study description
OBJECTIVE	Investigate the endocrine status of MERiT study subjects during IVF with MENOPUR vs rFSH alfa
COLLECTION AND HANDLING OF SERUM	Blood samples were obtained on Days 1 and 6 of stimulation, last stimulation day, and at oocyte retrieval. Follicular fluid was collected at retrieval from at least one follicle of ≥17 mm from which an oocyte had been retrieved
ASSESSMENT OF EMBRYO QUALITY	Embryo quality was assessed daily by local embryologists according to the number of blastomeres and the following morphology aspects: degree and localisation of fragmentation, blastomere uniformity, visual sign of multinucleation and cytoplasmic appearance
STIMULATION PROTOCOL AND TRANSFER	**See MERiT study description**

STUDY DESIGNS

MEGASET

MEGASET was a randomised, open-label, assessor-blind, parallel-group, multicentre non-inferiority trial

PATIENT POPULATION	749 patients, 21-34 years of age with regular ovulatory menstrual cycles and presumed to be ovulatory
RANDOMISATION	MENOPUR 150-375 IU/day (n=374) rFSH beta (Puregon) 150-375 IU/day (n=375)
PRIMARY OUTCOME	**Ongoing pregnancy rate per started cycle** (defined as presence of at least one intrauterine pregnancy with a viable foetus 10-11 weeks after embryo transfer in the fresh cycle)
SELECT SECONDARY OUTCOMES	Positive ß-hCG rate and clinical pregnancy rate (transvaginal ultrasound showing at least one intrauterine gestational sac with foetal heartbeat 5 to 6 weeks after transfer), follicular development, endocrine profile, oocytes retrieved, fertilisation rate, embryo quality, endometrial profile, the incidence of treatment-emergent adverse events (onset after start of stimulation), OHSS, pregnancy loss, and patient self-assessed local tolerability
STIMULATION PROTOCOL	**GnRH antagonist:** Patients were treated with 150 IU of gonadotrophin for 5 days, following which the stimulation could be adjusted by 75 IU and no more than 375 IU daily according to individual response
OVULATION TRIGGER	hCG (250 mcg)
TRANSFER	Fresh and frozen. A fresh single blastocyst transfer was performed on Day 5 after oocyte retrieval. Patients with no ongoing pregnancy by trial end and with a surplus of vitrified blastocysts could undergo replacement cycles with compulsory single blastocyst transfer within 1 year of randomisation

STUDY DESIGNS

BORDEWIJK META-ANALYSIS

Systematic review and meta-analysis of RCTs that compared rFSH with HP-hMG or HP-FSH for OS in couples undergoing IVF or ICSI up to July 2018. The meta-analysis aimed to assess the total amount of HP-hMG or rFSH used per woman that started an IVF/ICSI cycle

LITERATURE SEARCH	Medline, Embase and CINAHL databases up to July 2018 Search terms used were identical to the 2011 Cochrane review by Van Wely M, et al.
POPULATION	28 studies met the selection criteria, with 7,553 women included in total
HP-HMG TRIALS	• The search identified seven studies comparing HP-hMG with rFSH alfa or beta (n=3,397) and all provided data on the amount of gonadotrophin used - all seven studies administered MENOPUR as their HP-hMG • In total, 3,220 women (95%) started gonadotrophin treatment • All seven trials (MENOPUR versus rFSH) had data on clinical pregnancy and live birth rate and three trials (N=2,109) had data on cumulative live births following fresh and frozen embryo transfers

STUDY DESIGNS.
PI

STUDY DESIGNS

MEGASET-HR

The MEGASET-HR study was a multicentre, randomised, assessor-blind, non-inferiority study

PATIENT POPULATION	620 patients, 21-35 years of age, regular menstrual cycle and with serum anti Mullerian hormone (AMH) ≥5 ng/mL
RANDOMISATION	MENOPUR 150-300 IU/day (n=310) rFSH alfa (Gonal-F) 150-300 IU/day (n=309)
PRIMARY OUTCOME	**Ongoing pregnancy rate per started cycle** (defined by the presence of ≥1 intrauterine pregnancy with a detectable foetal heartbeat at 10 to 11 weeks of gestation)
SELECT SECONDARY OUTCOMES	Cumulative ongoing pregnancy, early pregnancy loss, OHSS frequency, follicular development, endocrine profile, endometrial development, number of oocytes retrieved, and blastocyst quality
STIMULATION PROTOCOL	**GRH antagonist:** Patients were treated with 150 IU daily gonadotrophin for 5 days, following which doses could be adjusted by 75 IU according to patient response
TRANSFER	Fresh and frozen. A fresh single blastocyst transfer was performed on Day 5 after oocyte retrieval. Patients with no ongoing pregnancy after the fresh cycle could undergo single frozen blastocyst transfers within 6 months of the date of randomisation

STUDIES OUTCOMES.

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EISG

MEGASET

MERITT

Bordewijk Meta-Analysis

MERiT Retrospective Analysis

MEGASET-HR

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STUDIES OUTCOMES.
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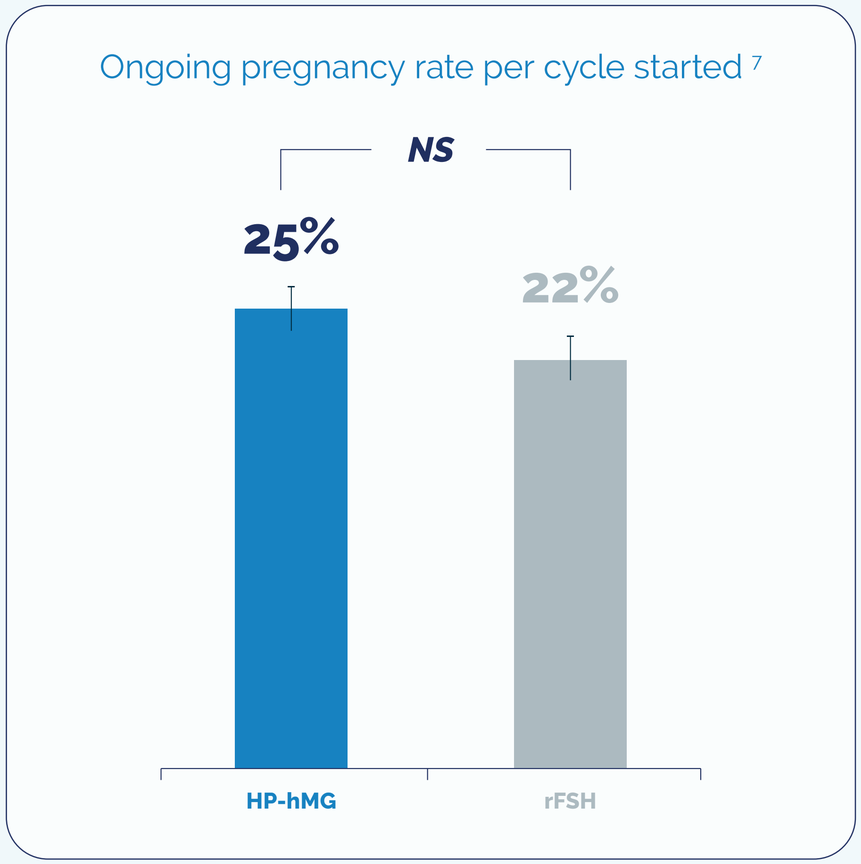
STUDIES

EISG: MENOPUR vs rFSH in IVF/ICSI
Primary end point: OPR*/cycle start



*OPR: Ongoing Pregnancy Rate

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STUDIES

EISG: MENOPUR vs rFSH in IVF/ICSI Safety

Adverse Events (≥0.3%) [7]

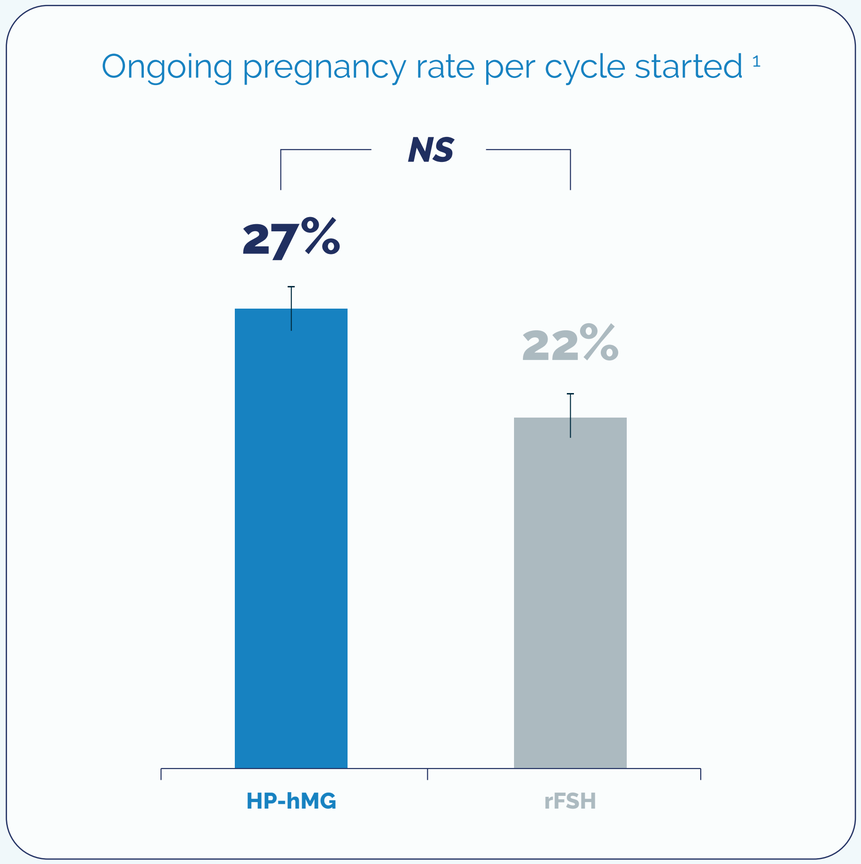
Symptom	MENOPUR (n=373)	rFSH (n=354)
Ovarian hyperstimulation	7%	5.1%
Abdominal pain	6.4%	7.1%
Injection site inflammation	4.8%	3.4%
Injection site pain	4.6%	3.7%
Headache	3.8%	2.8%
Enlarged abdomen	2.7%	0.6%
Nausea	1.9%	0.3%
Injection site reaction	1.3%	0.8%

STUDIES OUTCOMES.
PI

STUDIES

MERiT Study: MENOPUR vs rFSHα in IVF
Primary End Point: OPR*/cycle start

MERiT
MENOTROPHIN VS. RECOMBINANT FSH IN VITRO FERTILISATION TRIAL



*OPR: Ongoing Pregnancy Rate

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STUDIES

MERiT Study: MENOPUR vs rFSHα in IVF
Secondary measures

MERiT
MENOTROPHIN VS. RECOMBINANT FSH IN VITRO FERTILISATION TRIAL

Outcome [1]	MENOPUR (n=363)	rFSH (n=368)	*P* value
Clinical pregnancy rate (%)	28	24	*P*=NS
Endometrial thickness (mm), day of hCG	10.7 ± 1.9	10.8 ± 2.0	*P*=NS
Follicles ≥ 10 mm, day 6	4.1 ± 4.5	4.9 ± 4.9	*P*=0.007
Oocytes retrieved	10.0 ± 5.4	11.8 ± 5.7	*P*<0.001
Fertilization rate (%)	51.6 ± 29.2	52.5 ± 28.2	*P*=NS
Top-quality embryos (%)	11.3 ± 16.1	9.0 ± 13.0	*P*=0.044
E2 (pg/mL), day of hCG	1960.4 ± 1170.8	1797 ± 1089.1	*P*=0.031
P4 (ng/mL), day of hCG	0.82 ± 0.41	1.1 ± 0.53	*P*<0.001
Live birth rate* (%)	26	22	*P*=NS

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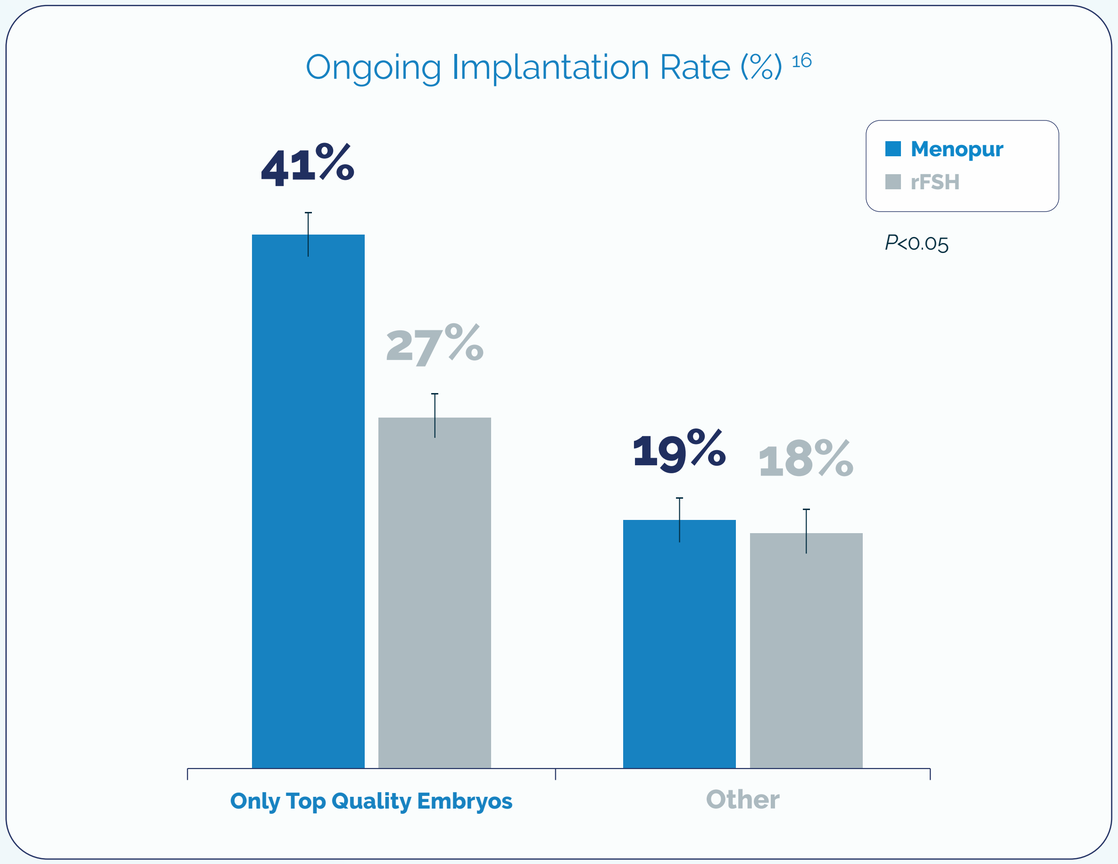
STUDIES

MERiT Study: MENOPUR vs rFSHα in IVF
Secondary measures: Ongoing Implantation Rate

MERiT
MENOTROPHIN VS. RECOMBINANT FSH IN VITRO FERTILISATION TRIAL



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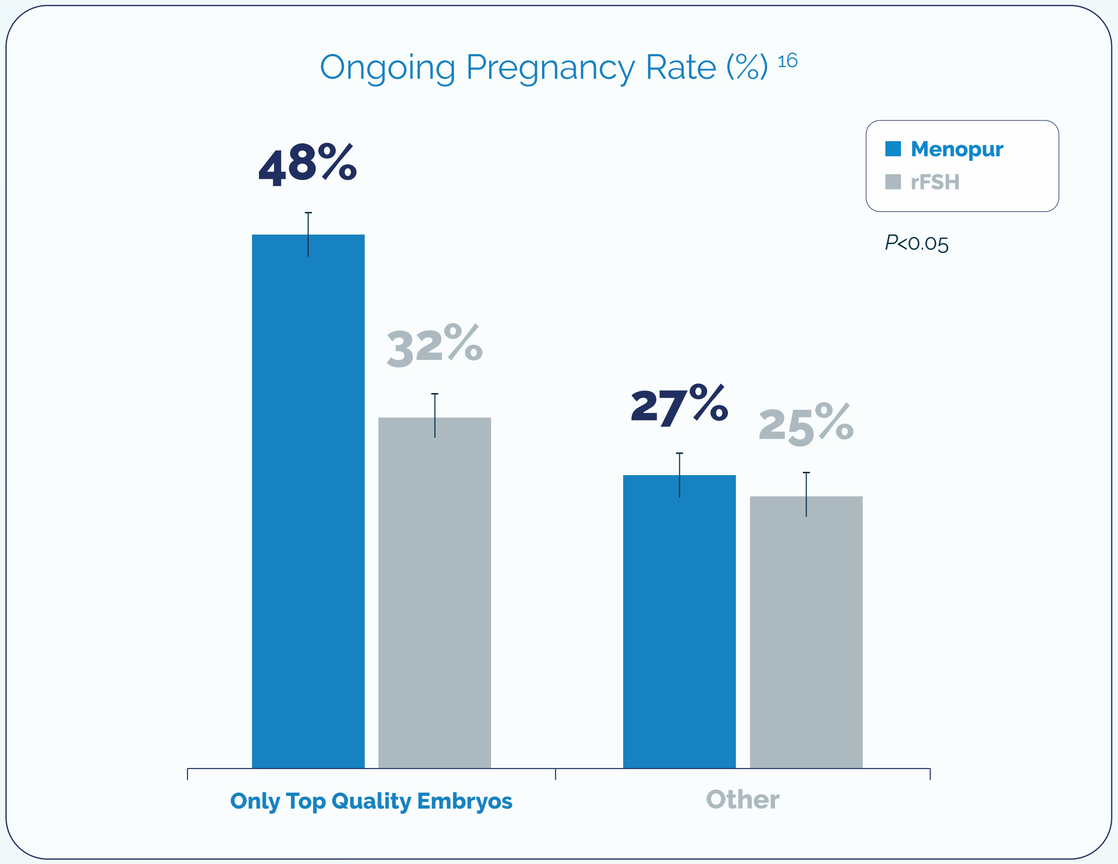
STUDIES

MERiT Study: MENOPUR vs rFSHα in IVF
Secondary measures: Ongoing Pregnancy Rate

MERiT
MENOTROPHIN VS. RECOMBINANT FSH IN VITRO FERTILISATION TRIAL



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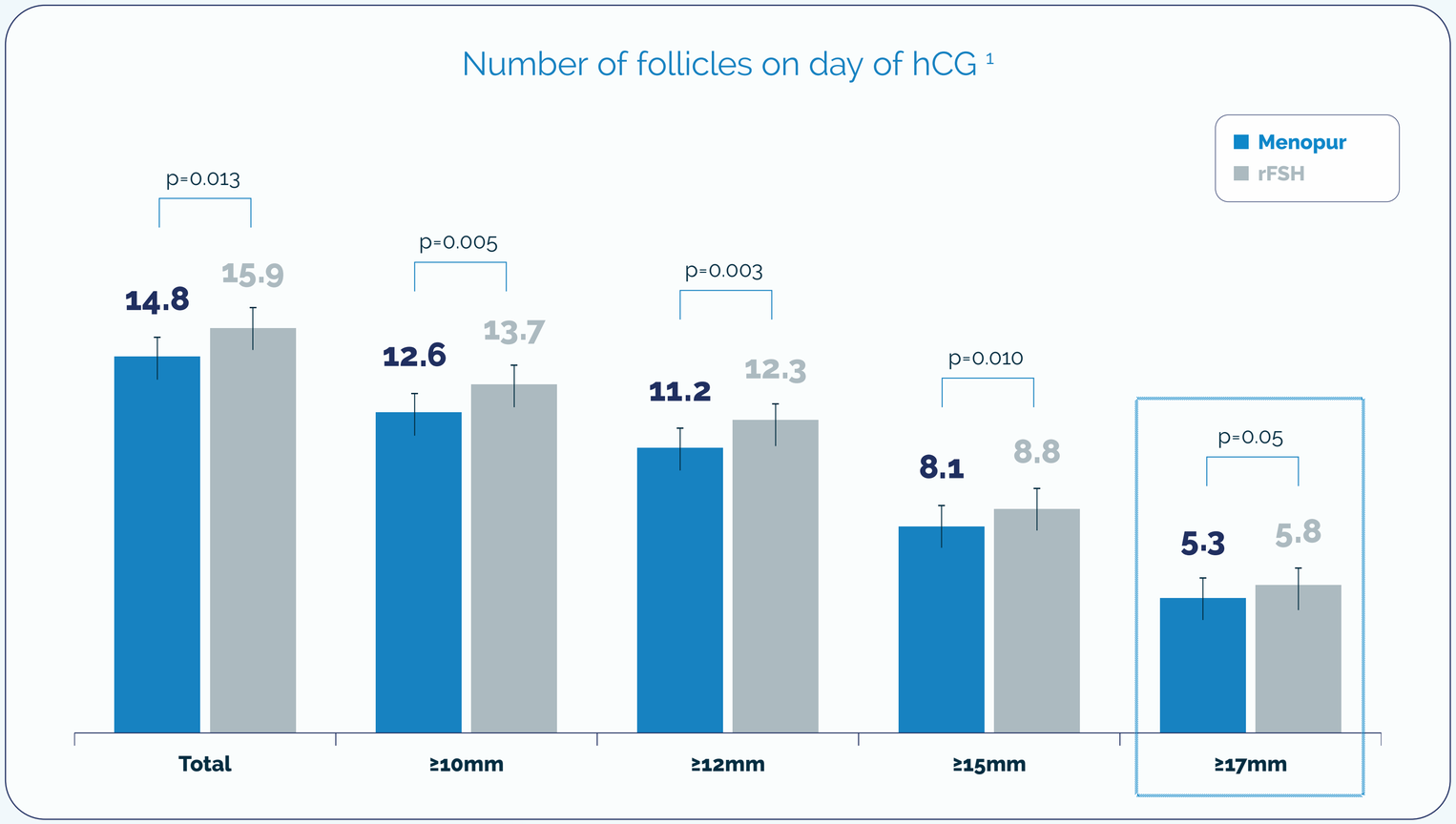
STUDIES

MERiT Study: MENOPUR vs rFSHα in IVF
Fewer small-sized follicles with Menopur compared with rFSH

MERiT
MENOTROPHIN VS. RECOMBINANT FSH IN VITRO FERTILISATION TRIAL



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STUDIES

MERiT Retrospective Analysis: MENOPUR vs rFSHα in IVF
Serum hormone levels [2]

Last day	FSH (IU/l)	LH (IU/l)	hCG (IU/l)	Estradiol (pg/mL)	*Progesterone (ng/mL)*
MENOPUR (n=363)	18.3 ± 6.0	1.8 ± 0.9	2.94 ±1.18	1960.4 ± 1170.8	*0.82 ± 0.41*
rFSH (n=368)	16.3 ± 4.7	1.7 ± 0.9	--	1797.0 ± 1089.1	*1.1 ± 0.53*
P value	P<0.001	P=NS	N/A	P=0.031	*P<0.001*

On last stimulation day, patients receiving MENOPUR demonstrated higher estradiol levels and lower progesterone levels than did patients receiving rFSH

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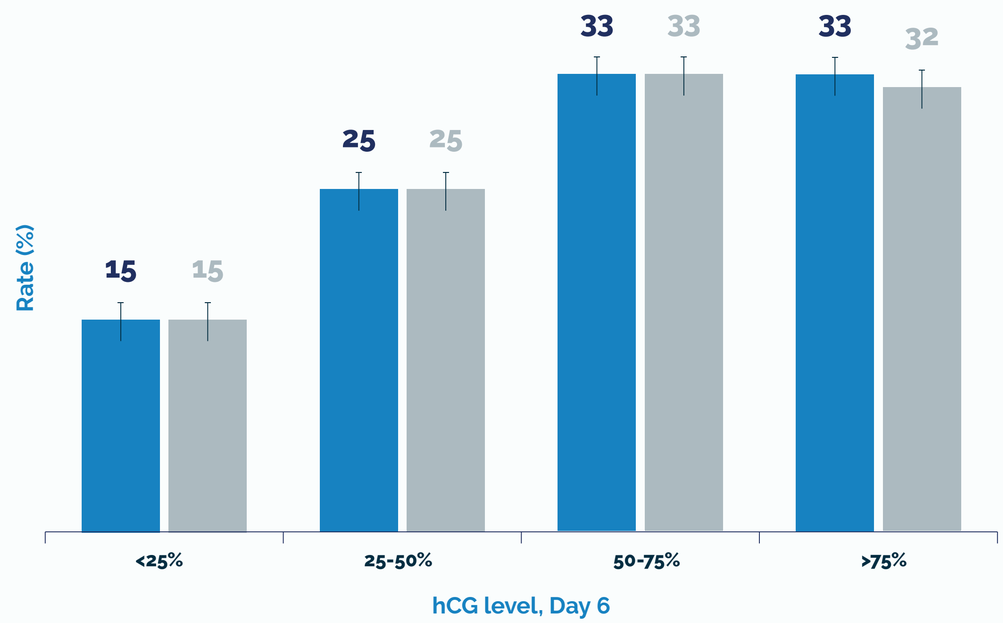
STUDIES

MERiT Retrospective Analysis: MENOPUR vs rFSHα in IVF
Day 6 serum hCG [2]



Day 6 serum hCG concentrations were positively correlated with pregnancy outcomes

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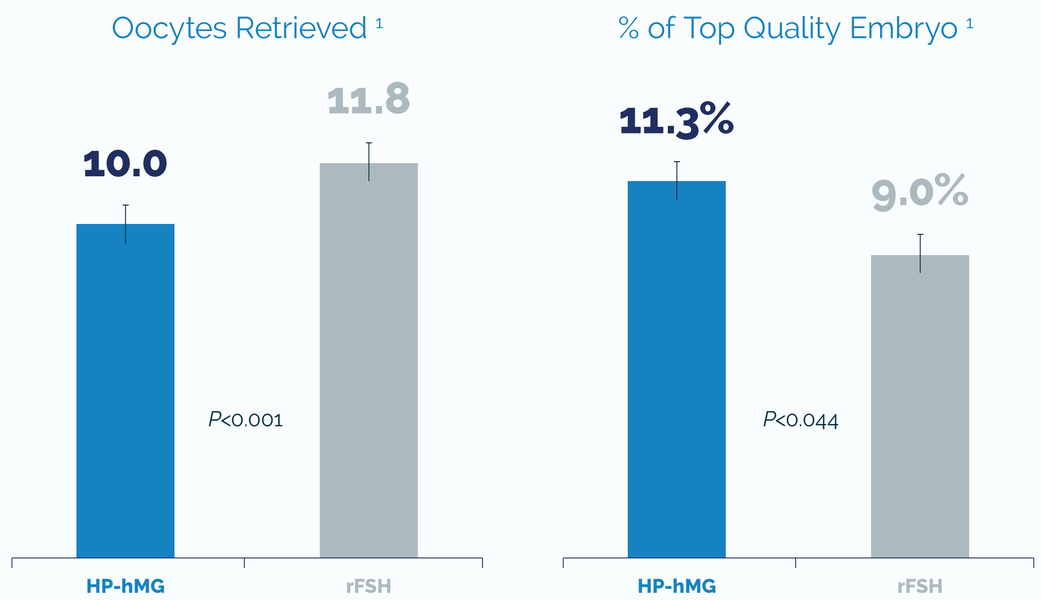
STUDIES

MERiT Retrospective Analysis:
Embryo analysis details: The story of 7.535 embryos!



Fewer oocytes retrieved with hMG, Greater proportion became top-quality embryos

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DREAMS THAT COME
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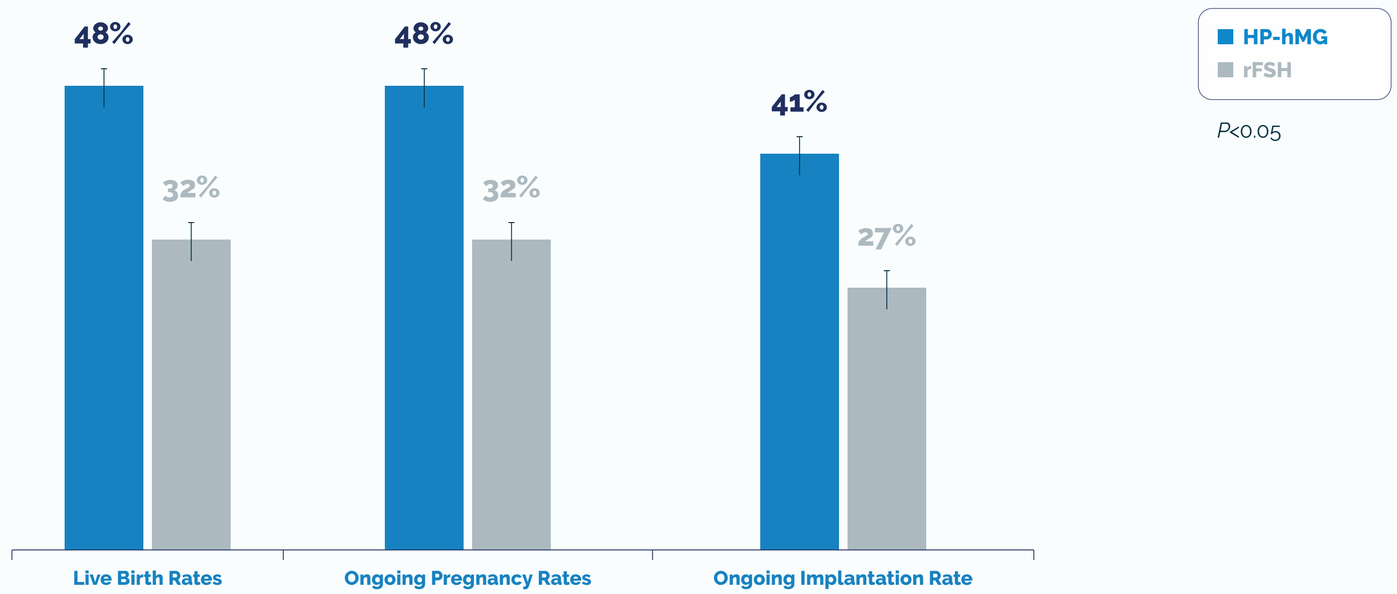
STUDIES

MERiT Retrospective Analysis:
Embryo analysis details: The story of 7.535 embryos!

Clinical Outcome (transfer of Top Quality Embryos) [16]



Fewer oocytes retrieved with hMG, Greater proportion became top-quality embryos

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DREAMS THAT COME
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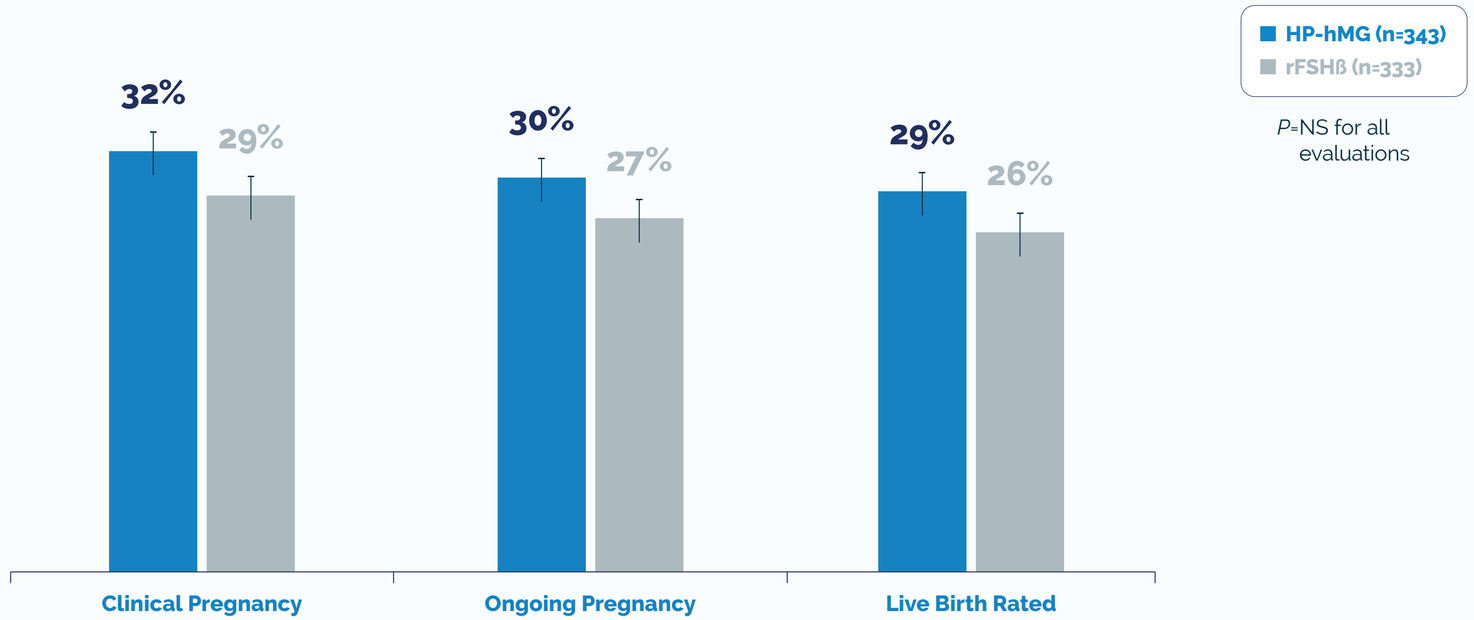
STUDIES

MEGASET Study: MENOPUR vs rFSHß in ICSI
Pregnancy rates per started cycle

megaset

Per Protocol Population [5]



No significant difference was observed in pregnancy rates between HP-hMG and rFSHß

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STUDIES OUTCOMES.
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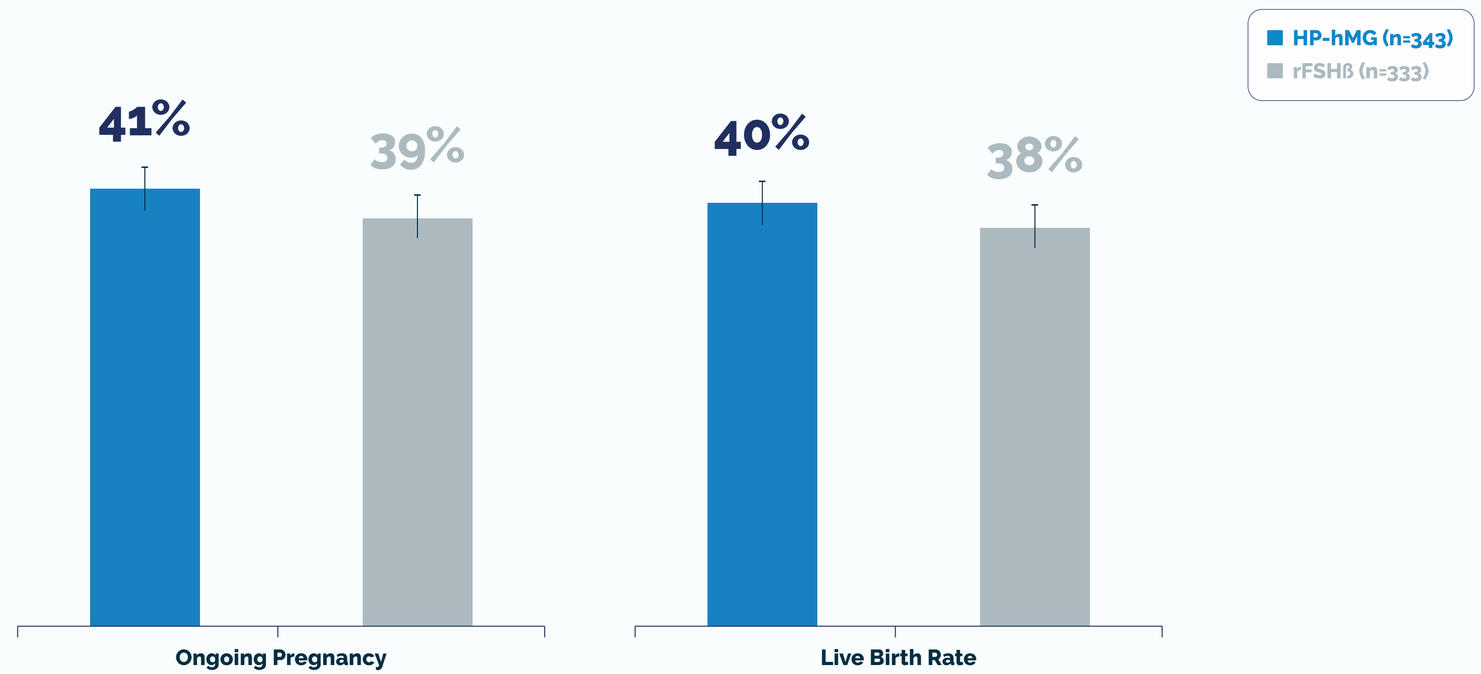
STUDIES

MEGASET Study: MENOPUR vs rFSHß in ICSI Cumulative Pregnancy & Live Birth Rates

megaset

Cumulative Outcomes [5]



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DREAMS THAT COME
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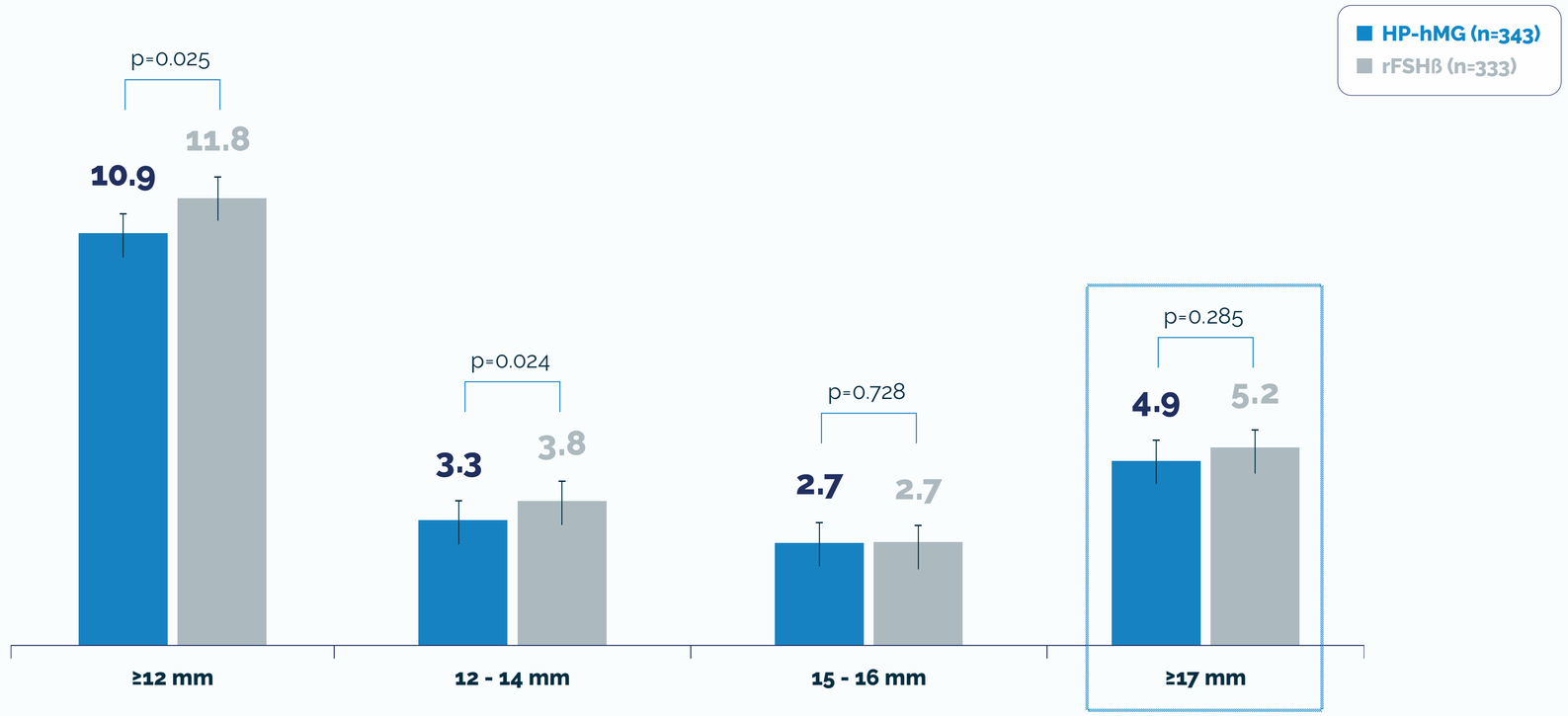
STUDIES

MEGASET Study: MENOPUR vs rFSHß in ICSI
Fewer small-sized follicles with MENOPUR compared with rFSHß

megaset

Number of follicles on day of hCG [5]



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DREAMS THAT COME
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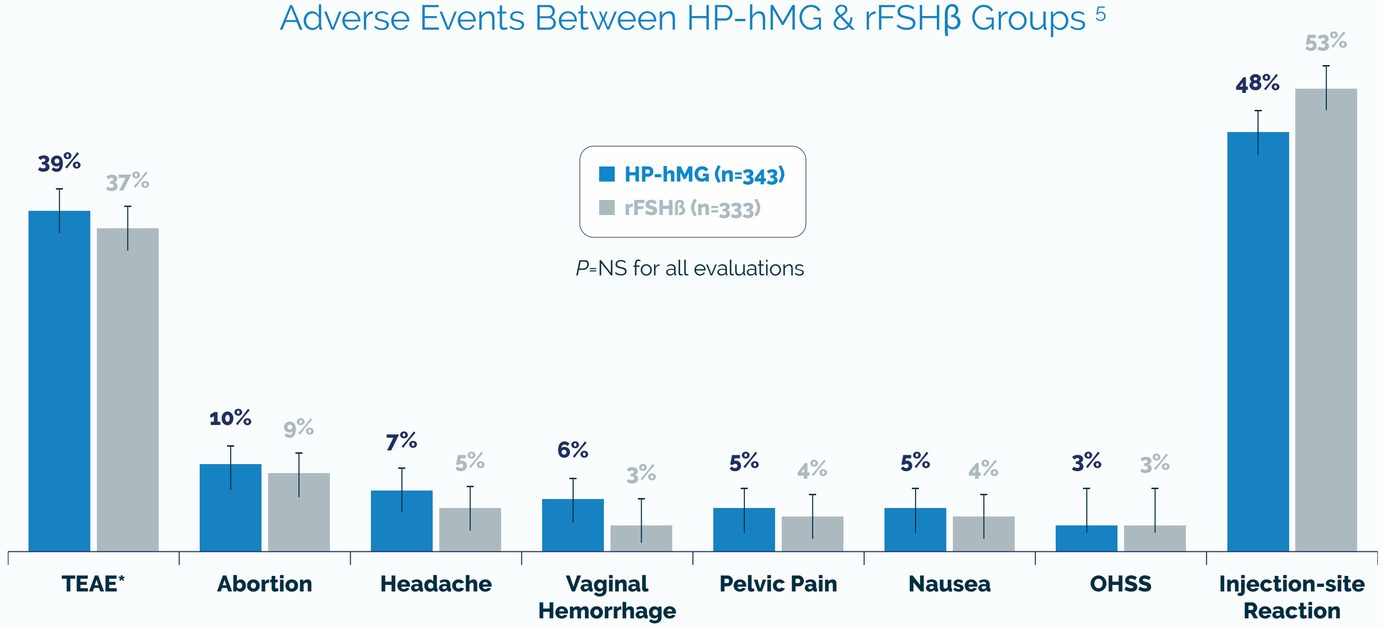
STUDIES

MEGASET Study: MENOPUR vs rFSHß in ICSI
Safety Data

megaset

Adverse Events Between HP-hMG & rFSHβ Groups [5]



* Treatment Emergent Adverse Events

Ovarian hyperstimulation syndrome occurred in 3% of patients in each group

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DREAMS THAT COME
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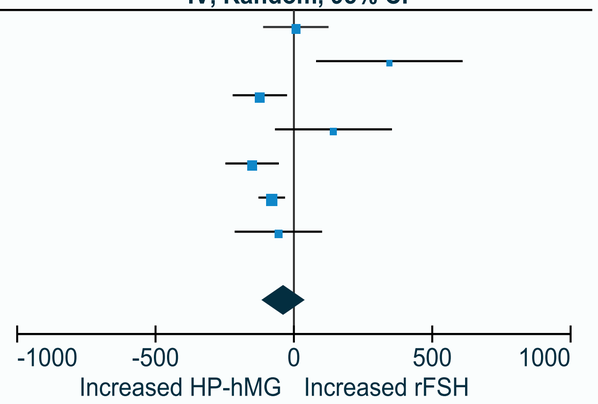
STUDIES

Bordweijk Meta-Analysis: Amount of gonadotrophins per woman that started IVF/ICSI cycle [8]

- **In 3,220 women included from 7 trials, there was no evidence of a difference in the amount of gonadotrophins (total dose) between HP-hMG and rec-FSH.**

	rFSH			HP-hMG				Mean Difference	
Study or Subgroup	Mean	SD	Total	Mean	SD	Total	Weight	IV, Random, 95% CI	Year
EISG 2002	2,775	810	354	2,768	817	373	15.5%	7.00 [-111.30, 125.30]	2002
Kilani 2003	2,025	795	50	1,680	530	50	6.4%	345.00 [80.16, 609.84]	2003
Andersen 2006	2,385	622	368	2,508	729	363	17.5%	-123.00 [-221.30, -24.70]	2006
Bosch 2008	2,624	801	140	2,481	994	140	8.7%	143.00 [-68.46, 354.46]	2008
Hompes 2008	1,781	468	256	1,932	628	250	17.6%	-151.00 [-247.68, -54.32]	2008
Devroey 2012	1,353	296	375	1,433	371	374	22.2%	-80.00 [-128.08, -31.92]	2012
Ye 2012	2,163	399	64	2,219	503	63	12.1%	-56.00 [-214.06, 102.06]	2012
Total (95% CI)			**1607**			**1613**	**100.0%**	**-37.15 [-115.02, 40.72]**	

Heterogeneity: Tau² = 6518.17; Chi² = 18.95, df = 6 (P = 0.004); I² = 68%
Test for overall effect: Z = 0.93 (P = 0.35)



STUDIES OUTCOMES.
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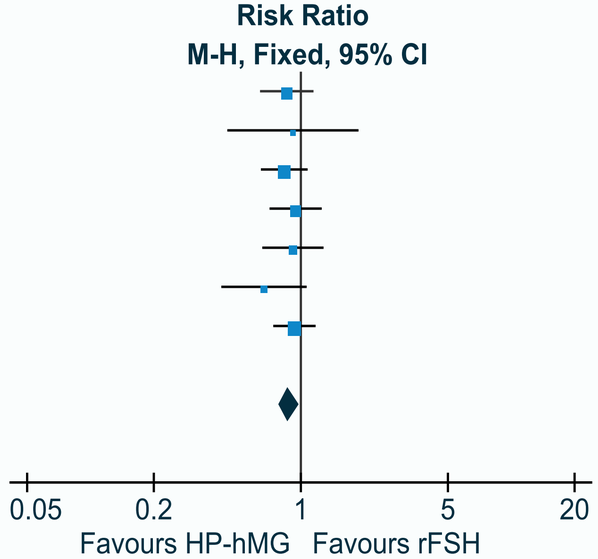
STUDIES

Bordweijk Meta-Analysis:
Clinical Pregnancy / Live Birth Rates [8]

- **In 3,397 women included from 7 trials, with a high quality of evidence, we found a significantly lower live birth rate in the rFSH stimulated women than in the HP-hMG stimulated women.**

Study or Subgroup	rFSH Events	rFSH Total	HP-hMG Events	HP-hMG Total	Weight	Risk Ratio M-H, Fixed, 95% CI	Year
EISG 2002	67	386	80	395	17.7%	0.86 [0.64, 1.15]	2002
Kilani 2003	11	50	12	50	2.7%	0.92 [0.45, 1.88]	2003
Andersen 2006	82	368	97	363	21.9%	0.83 [0.65, 1.08]	2006
Hompes 2008	71	317	74	312	16.7%	0.94 [0.71, 1.26]	2008
Bosch 2008	44	140	48	140	10.7%	0.92 [0.66, 1.28]	2008
Ye 2012	19	64	28	63	6.3%	0.67 [0.42, 1.07]	2012
Devroey 2012	100	375	107	374	24.0%	0.93 [0.74, 1.18]	2012
Total (95% CI)		**1700**		**1697**	**100.0%**	**0.88 [0.78, 0.99]**	
Total events	394		446				

Heterogeneity: Chi² = 2.08, df = 6 (P = 0.91); I² = 0%
Test for overall effect: Z = 2.13 (P = 0.03)



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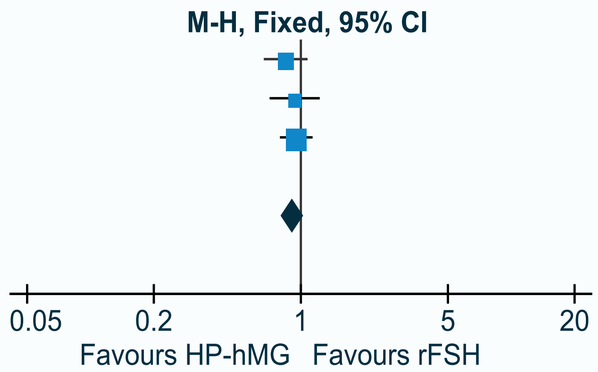
STUDIES

Bordweijk Meta-Analysis:
Cumulative Live Birth [8]

- **In 2,109 women included from 3 trials, with a high quality of evidence, We found insufficient evidence of a difference in cumulative live birth rate. The trend toward lower cumulative live birth rate continues.**

	rFSH		HP-hMG			Risk Ratio	
Study or Subgroup	**Events**	**Total**	**Events**	**Total**	**Weight**	**M-H, Fixed, 95% CI**	**Year**
Andersen 2006	91	368	106	363	31.7%	0.85 [0.67, 1.08]	2006
Hompes 2008	75	317	79	312	23.7%	0.93 [0.71, 1.23]	2008
Devroey 2012	143	375	150	374	44.6%	0.95 [0.80, 1.14]	2012
Total (95% CI)		**1060**		**1049**	**100.0%**	**0.91 [0.80, 1.04]**	
Total events	309		335				

Heterogeneity: Chi² = 0.60, df = 2 (P = 0.74); I² = 0%
Test for overall effect: Z = 1.38 (P = 0.17)



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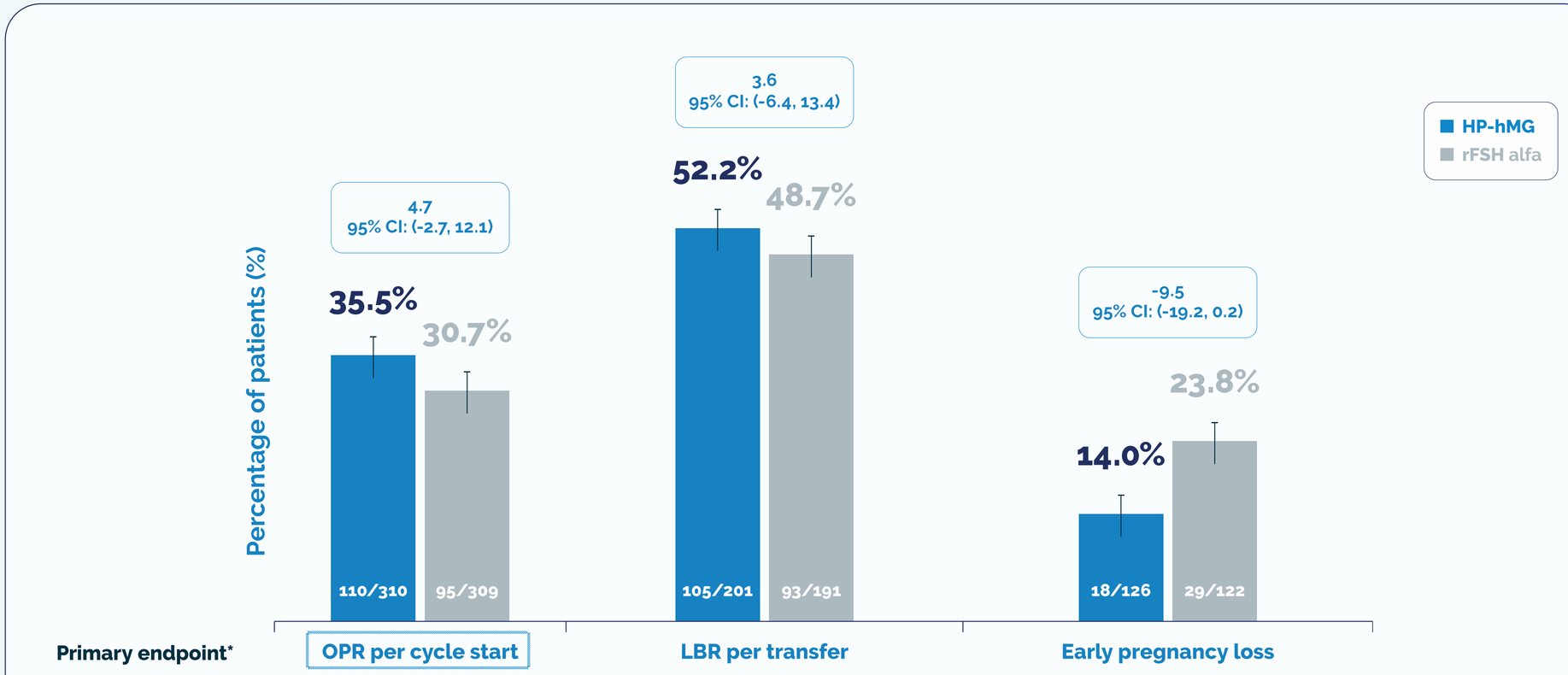
DREAMS THAT COME
FROM SCIENCE

STUDIES

MEGASET-HR study: MENOPUR vs rFSHα
Fresh cycle outcomes [6]

megaset-HR



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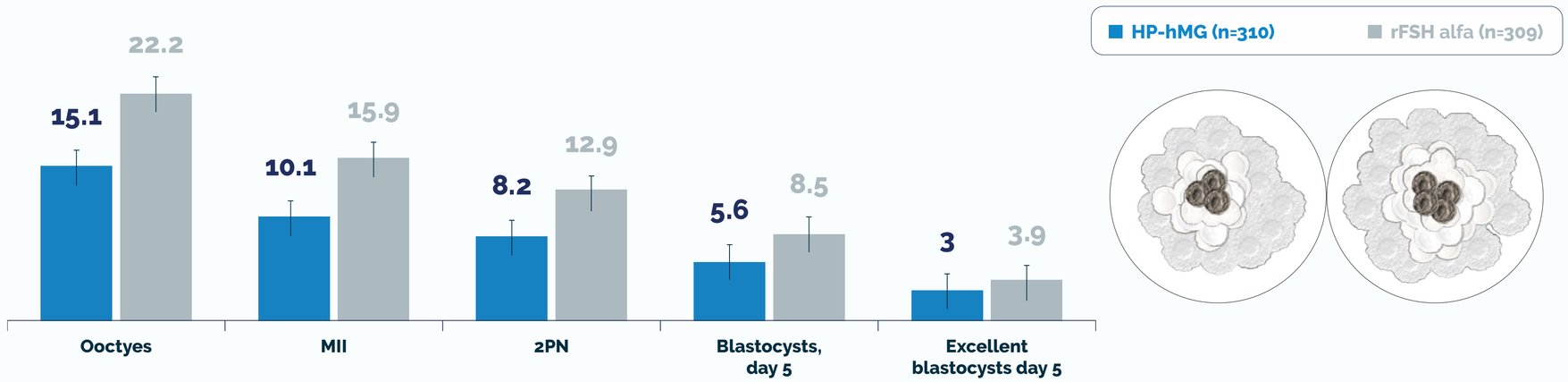
STUDIES

MEGASET-HR study: MENOPUR vs rFSHα
Post-retrieval laboratory parameters [6]

megaset-HR

Numerical difference:	-7.1	-5.8	-4.7	-2.9	-0.9
Hodges-Lehmann estimate difference (CI 95%)	-7.0 (-8.0, −5.0)	-5.0 (-7.0, −4.0)	-4.0 (-5.0, −3.0)	-3.0 (-3.0, −2.0)	0.0 (-1.0, 0.0)



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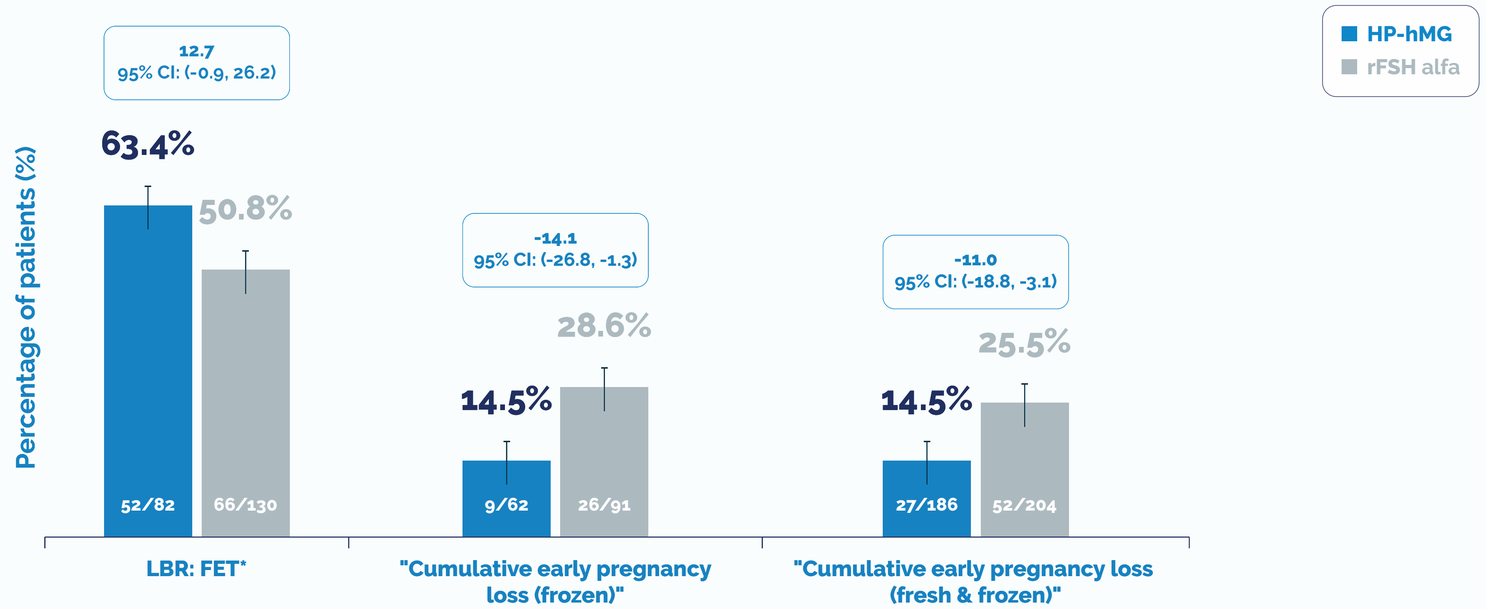
DREAMS THAT COME FROM SCIENCE

STUDIES

MEGASET-HR extension study:
FET outcomes [6]

megaset-HR



*LBR from all frozen transfers occurring within 6 months of randomisation (additive; max=3).

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DREAMS THAT COME
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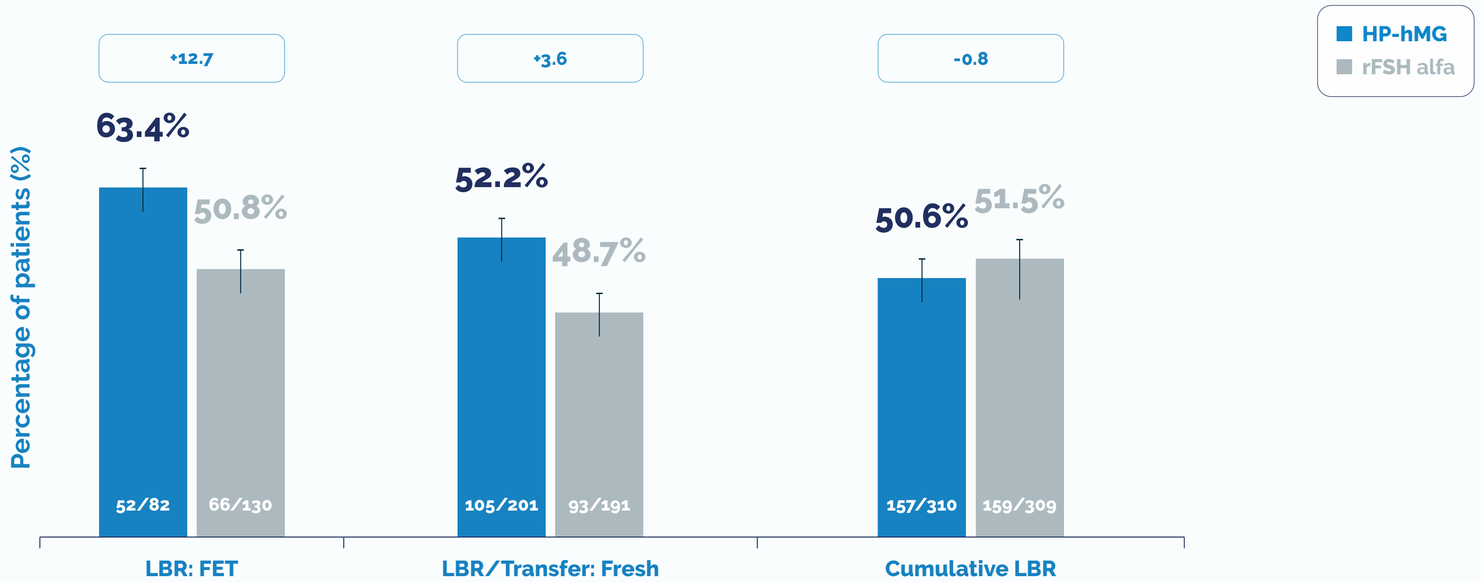
STUDIES

MEGASET-HR extension study:
FET outcomes [6]

megaset-HR



*LBR from all frozen transfers occurring within 6 months of randomisation (additive; max=3).

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DREAMS THAT COME
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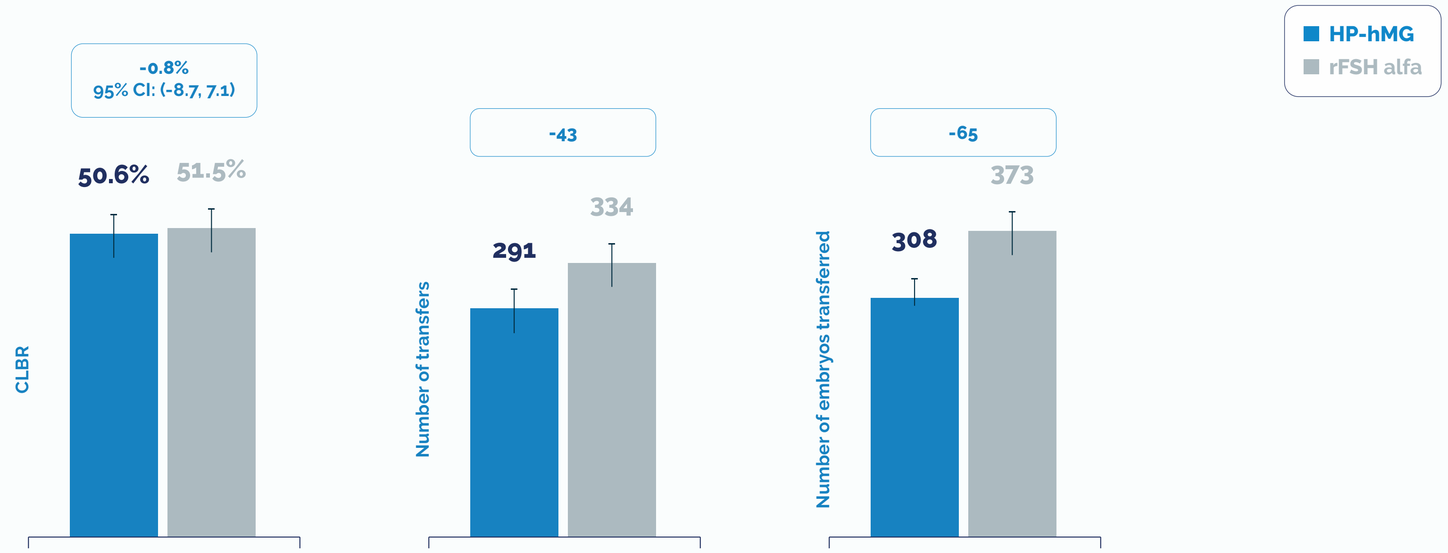
STUDIES

MEGASET-HR extension study: CLBR per transfer, number of transfers and number of embryos transferred [6]

megaset-HR

CLBR in 6 months is achieved with fewer transfers



Transfers involving multiple embryos were considered protocol deviations - all but 3 occurred with FET

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DREAMS THAT COME
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STUDIES

MEGASET-HR study: Safety
Overall safety [6]

megaset-HR

Parameter1	HP-hMG n=310)		rFSH alfa (n=309)		Total (n=619)	
TEAE	179	(58%)	218	(71%)	**397**	**(64%)**
Procedural pain	71	(23%)	71	(23%)	**142**	**(23%)**
Nausea	37	(12%)	40	(13%)	**77**	**(12%)**
Abdominal distension	25	(8%)	35	(11%)	**60**	**(10%)**
Constipation	22	(7%)	36	(12%)	**58**	**(9%)**
Headache	29	(9%)	22	(7%)	**51**	**(8%)**
Abdominal pain	21	(7%)	24	(8%)	**45**	**(7%)**
Vaginal haemorrhage	21	(7%)	22	(7%)	**43**	**(7%)**
OHSS*	30	(10%)	66	(21%)	**96**	**(16%)**
Mild	7	(2%)	18	(6%)	**25**	**(4%)**
Moderate	15	(5%)	39	(13%)	**54**	**(9%)**
Severe	8	(3%)	9	(3%)	**17**	**(3%)**
Serious AEs	8	(3%)	11	(4%)	**19**	**(3%)**
AE-related discontinuation	2	(1%)	4	(1%)	**6**	**(1%)**
Deaths	0		0		**0**	

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DREAMS THAT COME
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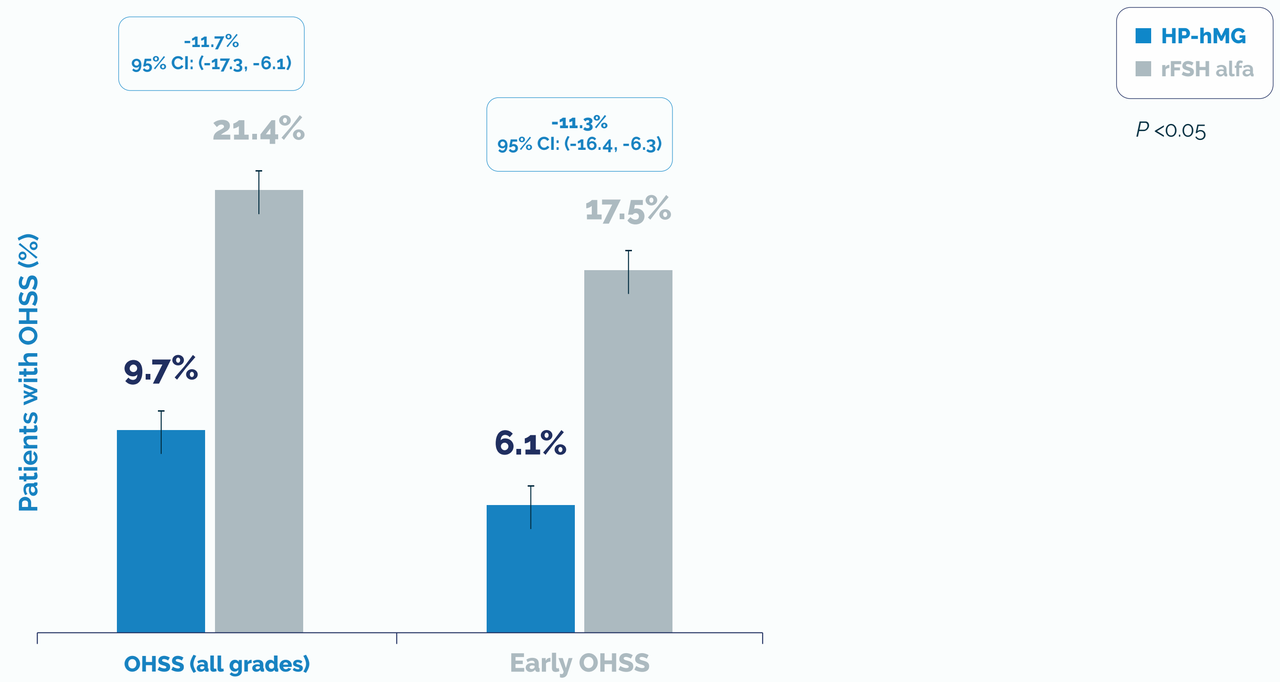
STUDIES

MEGASET-HR study: Safety
Rate of OHSS [6]

megaset-HR



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References

*Gonal-F [1,4,7]

**MENOPUR contains 9.9 IU of hCG and 0.4 IU of LH, which is equivalent to 75 IU of LH bioactivity.[15]

†Puregon [5]

***MEGASET-HR only included patients predicted to be high responders (serum AMH ≥35.7 pmol/L (5.2 ng/mLl). [6]

‡Classification of OHSS in MERiT, MEGASET, and MEGASET-HR was determined using Golan's classification system. [1,5,6]

#The primary outcome in all listed RCTs was ongoing pregnancy rate. [1,4-7]

§Live birth rate was either a secondary or post-trial outcome from non-inferior phase 3/4 RCTs. EISG, MERIT, MEGASET and MEGASET-HR were neither designed nor powered to assess results based on secondary outcomes and no final conclusion can be derived from those results. [1,4-6]

¶Top-quality embryo was defined as 4-5 blastomeres on Day 2, 7 or more blastomeres on Day 3, equally-sized blastomeres and ≤20% fragmentation on Day 3 and no multi-nucleation. MENOPUR (n=363); rFSH alfa (n=368). Embryo quality was assessed daily by local embryologists according to the number of blastomeres and the following morphology aspects: degree and localisation of fragmentation, blastomere uniformity, visual sign of multinucleation and cytoplasmic appearance. Proportion of top quality embryos was a secondary outcome. [1]

References

‖Safety was a secondary outcome from non-inferior phase 3/4, RCTS. [1,5-7]

AMH: anti Mullerian hormone
RCT: randomised controlled trial
rFSH: recombinant follicle stimulating hormone.
APT: all patients treated
GnRH: gonadotrophin-releasing hormone
ITT: intent-to-treat
IU: international unit
LBR: live birth rate
mITT: modified intent-to-treat
CI: confidence interval
ICSI: intracytoplasmic sperm injection
IVF: in vitro fertilisation
OPR: ongoing pregnancy rate
HP-hMG: highly purified human menotrophin
I: interval variable
FET: frozen embryo transfer
GRH: gonadotrophin-releasing hormone
LBR: live birth rate
ART: assisted reproductive technology
FSH: follicle stimulating hormone
hCG: human chorionic gonadotrophin
LH: luteinising hormone
OHSS: ovarian hyperstimulation syndrome

STUDY DESIGNS

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Menopur®
menotrophin

FERRING PHARMACEUTICALS

Menopur 75 IU & Menopur 600 IU & 1200 IU APIs:

MENOPUR® Powder and solvent for solution for injection

Vials of powder and solution for injection. Contains highly purified menotrophin corresponding to FSH activity and LH activity. **Indications:** Treatment of infertility for: 1. women with anovulation 2. women who have been unresponsive to treatment with clomiphene citrate. 3. Controlled ovarian hyperstimulation. Menopur® 600IU and 1200IU can also treat infertility in: 1. women undergoing assisted reproduction 2. Female sterility with hypo- or normogonadotrophic ovarian insufficiency. 3. Men with hypo- or normogonadotrophic hypogonadism, spermatogenesis stimulation. **Posology and method of Administration:** S.C. Menopur® 75IU: S.C. + I.M. The recommended initial dose of Menopur®varies from 75IU-225IU.Maximum doses should not exceed 225IU-450IU. Recommendations about dosage and duration of treatment may change depending on the actual treatment protocol. **Contraindications:** MENOPUR® is contraindicated in women who have: tumors of the pituitary gland or hypothalamus, ovarian, uterine or mammary carcinoma, gynecological hemorrhage of unknown etiology, hypersensitivity to the active substance or excipients, premature menopause, ovarian cysts or enlarged ovaries not due to PCOD, primary ovarian failure, malformation of sexual organs incompatible with pregnancy, and fibroid tumors of the uterus incompatible with pregnancy. MENOPUR® is also contraindicated in men who have prostatic carcinoma, and testicular tumors. **Special Population:** Menopur® is contraindicated for pregnant and lactating women. **Undesirable effects:** Abdominal pain, nausea, abdominal distention, injection site reaction and pain, headache, OHSS, and pelvic pain. **Storage Conditions:** Menopur® 600 and 1200IU: Store in a refrigerator (2˚C - 8˚C). Do not freeze. After reconstitution, the solution may be stored for a maximum of 28 days at not more than 25˚C. Menopur® 75IU: Do not store above 30˚C. Do not freeze. To be used immediately after reconstitution. **Pack Size:** Menopur® 600IU and 1200IU: packs of 1 injection set. Menopur® 75IU: packs of 5 or 10 injection sets. Not all packs may be marketed. **Prescription status**: Prescription only. For further details on the medicinal product; please refer to the package insert leaflet. To report side effect: Call at +962-6-5501045 or send an E-mail to: JOo-QAME@ferring.com.For the Kingdom of Saudi Arabia: The National Pharmacovigilance (NPC), SFDA Call Center: 19999, Email: npc.drug@sfda.gov.sa, Website: https://ade.sfda.gov.sa/

MENOPUR® Powder and solvent for solution for injection

Vials of powder and solution for injection. Contains highly purified menotrophin corresponding to FSH activity and LH activity. **Indications:** Treatment of infertility for: 1. women with anovulation 2. women who have been unresponsive to treatment with clomiphene citrate. 3. Controlled ovarian hyperstimulation. Menopur® 600IU and 1200IU can also treat infertility in: 1. women undergoing assisted reproduction 2. Female sterility with hypo- or normogonadotrophic ovarian insufficiency. 3. Men with hypo- or normogonadotrophic hypogonadism, spermatogenesis stimulation. **Posology and method of Administration:** S.C. Menopur® 75IU: S.C. + I.M. The recommended initial dose of Menopur®varies from 75IU-225IU.Maximum doses should not exceed 225IU-450IU. Recommendations about dosage and duration of treatment may change depending on the actual treatment protocol. **Contraindications:** MENOPUR® is contraindicated in women who have: tumors of the pituitary gland or hypothalamus, ovarian, uterine or mammary carcinoma, gynecological hemorrhage of unknown etiology, hypersensitivity to the active substance or excipients, premature menopause, ovarian cysts or enlarged ovaries not due to PCOD, primary ovarian failure, malformation of sexual organs incompatible with pregnancy, and fibroid tumors of the uterus incompatible with pregnancy. MENOPUR® is also contraindicated in men who have prostatic carcinoma, and testicular tumors. **Special Population:** Menopur® is contraindicated for pregnant and lactating women. **Undesirable effects:** Abdominal pain, nausea, abdominal distention, injection site reaction and pain, headache, OHSS, and pelvic pain. **Storage Conditions:** Menopur® 600 and 1200IU: Store in a refrigerator (2˚C - 8˚C). Do not freeze. After reconstitution, the solution may be stored for a maximum of 28 days at not more than 25˚C. Menopur® 75IU: Do not store above 30˚C. Do not freeze. To be used immediately after reconstitution. **Pack Size:** Menopur® 600IU and 1200IU: packs of 1 injection set. Menopur® 75IU: packs of 5 or 10 injection sets. Not all packs may be marketed. **Prescription status**: Prescription only. For further details on the medicinal product; please refer to the package insert leaflet. To report side effect: Call at +962-6-5501045 or send an E-mail to: JOo-QAME@ferring.com. For the Kingdom of Saudi Arabia: The National Pharmacovigilance (NPC), SFDA Call Center: 19999, Email: npc.drug@sfda.gov.sa, Website: https://ade.sfda.gov.sa/

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